NIHR | National Institute for Health and Care Research

Summary

Reference Number	NIHR304452
Lead Applicant	Dr Barbora Silarova
Research Title	Healthy you – keeping yourself healthy while supporting someone living with dementia: development and feasibility testing of a complex intervention
Plain English Summary	**Aim(s) of the research:** The focus of this Fellowship is on family members and friends who support someone living with dementia (unpaid carers). Eight former and current unpaid carers highlighted that they would like to understand how they can increase their physical activity, improve their diet and manage their time better. Therefore, as a first step, we will prioritise which concern out of these three is the most important for unpaid carers living across England. As a second step, we will then design an intervention that will help unpaid carers to address the concern selected in the first step. As a last step, we will test whether the developed intervention is practical and acceptable for unpaid carers and for those who may be delivering it. **Background to the research:** Across the UK there are 700,000 family members and friends who support a person living with dementia. In the academic literature they are referred to as carers, unpaid carers, family carers or supporters etc. but the family members and friends may not use any of these terms to describe themselves. As a result of their caring responsibilities, carers are often unable to do things that would support their own health and wellbeing. Unpaid carers themselves highlighted that '*It is exhausting to be a carer, it takes so much out of you*'. Currently, we know very little about how we can support unpaid carers to increase their physical activity, improve their diet and manage their time better. **Design and methods used:** In this Fellowship we will follow a step by step guide called the Behaviour Change Wheel to understand how we can support unpaid carers to improve their health and wellbeing. In Stage 1 we will conduct a needs assessment (through reviewing existing evidence and collecting information among 100 unpaid carers living in England) to identify whether we should focus on e.g. helping carers to manage their time better (to have 'me time'). In Stage 2 we will use a range of methods to design the intervention itself. This will include a consensus exercise with 30 unpaid carers to define the exact focus of the intervention. We will then conduct five focus groups (3-5 people per group) with unpaid carers to gain further insight into how the intervention should look. We will then design the intervention itself and share it with five focus groups (3-5 people per group) including e.g. carers, health and social care professionals etc. to gather their feedback. In Stage 3 we will test whether the proposed intervention is practical and acceptable by the unpaid carers and the people who may deliver it. **Patient/service user, carer and public involvement:** Unpaid carers played a central role in co-producing this Fellowship application. Over a period of twelve months we met on a monthly basis to decide together the focus of the Fellowship, how we are going to answer our research questions, disseminate results etc. We were a team of ten people - eight people self-identified as a former or current unpaid carer living across England and Scotland. They will continue to work on the Fellowship as co-researchers – this means they will be part of the core research team, they will jointly own the research and be responsible for it. **Dissemination:** Unpaid carers highlighted that it is important that the results from this Fellowship reach a wide range of people including carers, health and social care professionals and that the results are shared throughout the Fellowship regularly. We will use a range of approaches, for example: Podcast/YouTube channel; articles; and working with an illustrator to capture some of the findings.

Our purpose for collecting this data is to communicate with you about your application and have the necessary information to evaluate you for a grant. The data we collect here is collected in the public interest. Information provided here may be subject to Freedom of Information requests.

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1. Application Summary Information

Host organisation (which will administer any award)
University of Kent

Partner organisation (if applicable)	

NIHR Academy Strategic Theme	Social care

Partnership Fellowship	N/A

Advanced Fellowship application type	Dementia (NIHR Highlight Notice)

Research Title	Healthy you – keeping yourself healthy while supporting someone living with dementia: development and feasibility testing of a complex intervention

Research Type	Feasibility study, Qualitative study, Survey

Proposed start date if grant awarded	01/01/2025

Do you wish to hold an Advanced Fellowship for the full time equivalent of 2, 3, 4 or 5 years?
2

WTE %	0.55

Duration The duration in months below for your award is based on your choice of years for the Advanced Fellowship and your chosen WTE%
44

2. Applicant CV

Degrees and Professional Qualifications

From	To	Qualification	Subject	Country	Institution	Class	Department
09/2011	08/2013	MSc	Masters in Clinical and Health Psychology (research)	Netherlands	Leiden University	NA	The Institute of Psychology
10/2009	01/2014	PhD	Unraveling the role of sense of coherence in coronary heart disease patients	Netherlands	University of Groningen	NA	Faculty of Medical Sciences
09/2004	05/2009	MSc	Masters in Psychology (Clinical, Counselling and Educational psychology)		University of Trnava	Cum Laude	Department of Psychology

Present and previous positions
(including from/to dates, job title and employer)

From	To	Position	Organisation	WTE % Research
01/2024	-	ARC KSS Dementia Research Fellow	Centre for Health Services Studies, University of Kent	100
10/2023	12/2023	Research Fellow	Personal Social Services Research Unit, University of Kent	100
01/2022	03/2024	Research Advisor	NIHR, Research Design Service, South East	10
09/2019	09/2023	Research Associate	Personal Social Services Research Unit, University of Kent	100
09/2017	08/2019	Research Fellow	Department of Psychology, University of Exeter	100
01/2017	03/2017	Postdoctoral Fellow	Institute for Health & Aging, University California San Francisco	100
01/2017	06/2018	Visiting Researcher	MRC Epidemiology Unit, University of Cambridge	100
02/2014	12/2016	Career Development Fellow	MRC Epidemiology Unit, University of Cambridge	100
09/2012	03/2013	Research Assistant	PsyQ, The Hague	5
10/2011	12/2011	Research Assistant	Institute of Psychology, Leiden University	5
10/2009	01/2014	PhD candidate	Department of Community & Occupational Health, University of Groningen	100

Research Grants held

Title of Grant	Role in the Research Grant	Total Grant £	Source of Grant (Organisation Name)	Duration (In Months)	Start Date of Funding	End Date of Funding
Healthy lifestyle support for carers of people living with dementia	Lead Applicant: conceptualization, investigation, methodology, project administration,	32166	Three NIHR Research Schools' Dementia	17	01/07/2022	18/01/2023

Title of Grant	Role in the Research Grant	Total Grant £	Source of Grant (Organisation Name)	Duration (In Months)	Start Date of Funding	End Date of Funding
	writing-original draft The main aim of this award was to develop this NIHR Advanced Fellowship aplication that has been co-produced together with eight carers (family members) of people living with dementia.		Programme			
Mind the gap: Implications of the digital divide on healthcare inequalities during the NHS digital transformation	Co-Investigator: conceptualization, methodology; writing-review and editing Members of the team: Ben Ainsworth (Lead co-applicant); Charlotte Dack (Lead co-applicant), Sabrina Grant; Gemma Lasseter; Sophie Turnbull; Kate Binnie; Laura Colebrooke The aim of this research was to understand how and when healthcare professionals are using digital healthcare tools with patients.	4974	GW4 Crucible Seed Funding	14	01/07/2019	01/09/2020
Happy to Share Cards (HtSC)	Co-applicant: conceptualization, methodology, writing-review and editing Team members: Donna Poade (Lead Applicant); Dr Ruth Lamont The present study explored the use of HtSC among early adopters of this initiative and is the first evaluation of their use.	3000	ESRC Impact Acceleration Account (IAA)	12	01/06/2019	01/06/2020
Keep Your Brain Health in Check: The KLICK project	Lead applicant: conceptualization, data curation, investigation, methodology, project administration, writing-original draft Members of the team: Rachel Collins; Lisa Gibbons; James Coe	9990	ESRC Impact Acceleration Account (IAA)	9	01/10/2018	31/07/2019

Title of Grant	Role in the Research Grant	Total Grant £	Source of Grant (Organisation Name)	Duration (In Months)	Start Date of Funding	End Date of Funding
	This project aimed to review how the Monsenso Solution can be used to support individuals to engage in healthy behaviours to reduce their risk of getting dementia.					

Publication Record

(include the list of authors, article title, journal name, and volume)
This information can be edited via the "My Research Outputs" page of your ARAMIS account.

Publications
James Caiels, **Silarova, B.**, Milne, A. and Beadle-Brown . (2023) 'Strengths-based Approaches—Perspectives from Practitioners', *The British Journal of Social Work*.
Silarova, B., Rand, S., Towers, AM. and Jones, K.. (2023) 'Feasibility, validity and reliability of the ASCOT-Proxy and ASCOT-Carer among unpaid carers of people living with dementia in England.', *Health and quality of life outcomes*, 21(1).
Rand, S., Collins, G., Zhang, W., Milne, A. and **Silarova, B.**. (2023) 'Applying outcomes in community-based social care practice in England', *Journal of Long Term Care*.
Rand, S., Towers, A., Malley, J. and **Silarova, B.**. (2023) 'Exploring the structural characteristics of the Adult Social Care Outcomes Toolkit (ASCOT) and ASCOT-Carer', *NIHR open research*, 21(2).
Zhang, W., Rand, S., Milne, A., Collins, G. and **Silarova, B.**. (2022) 'The quality of life of older carers and the people they support: An international scoping review.', *Health & Social Care in the Community*, 30(6).
Rand, S., Zhang, W., Collins, G., **Silarova, B.** and Milne, A.. (2022) 'Applying a dyadic outcomes approach to supporting older carers and care-recipients: A qualitative study of social care professionals in England.', *Health & Social Care in the Community*, 30(6).
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Rand, SE., **Silarova, B.**, Towers, AM. and Jones, K.. (2021) 'Social care-related quality of life of people with dementia and their carers in England.', *Health & Social Care in the Community*, 30(5).
Oliveira, D., Deckers, K., Zheng, L., Macpherson, H., Ishak, WS. and **Silarova, B.**. (2021) 'The career development of early- and mid-career researchers in dementia should be a global priority: a call for action.', *Aging & mental health*, 26(3).
Sabatini, S., **Silarova, B.**, Martyr, A., Collins, R., Ballard, C., Anstey, KJ., Kim, S. and Clare, L.. (2020) 'Associations of Awareness of Age-Related Change With Emotional and Physical Well-being: A Systematic Review and Meta-analysis.', *The Gerontologist*, 60(6).
Bayne, M., Fairey, M., **Silarova, B.**, Griffin, SJ., Sharp, SJ., Klein, WMP., Sutton, S. and Usher-Smith, JA.. (2019) 'Effect of interventions including provision of personalised cancer risk information on accuracy of risk perception and psychological responses: A systematic review and meta-analysis.', *Patient Education and Counseling*, 103(1).
Silarova, B., Sharp, S., Usher-Smith, JA., Lucas, J., Payne, RA., Shefer, G., Moore, C., Girling, C., Lawrence, K., Tolkien, Z., Walker, M., Butterworth, A., Di Angelantonio, E., Danesh, J. and Griffin, SJ.. (2019) 'Effect of communicating phenotypic and genetic risk of coronary heart disease alongside web-based lifestyle advice: the INFORM Randomised Controlled Trial.', *Heart (British Cardiac Society)*, 105(13).
Collins, R., **Silarova, B.** and Clare, L.. (2019) 'Dementia Primary Prevention Policies and Strategies and Their

Publication Record

(include the list of authors, article title, journal name, and volume)
This information can be edited via the “My Research Outputs” page of your ARAMIS account.

Publications
Local Implementation: A Scoping Review Using England as a Case Study.’, *Journal of Alzheimer's disease : JAD*, 70(s1).
Wou, C., **Silarova, B.**, Griffin, S. and Usher-Smith, JA.. (2018) ‘The associations between the response efficacy and objective and subjective change in physical activity and diet in the Information and Risk Modification trial.’, *Public health*, 165.
Silarova, B., Nelis, SM., Ashworth, RM., Ballard, C., Bieńkiewicz, M., Henderson, C., Hillman, A., Hindle, JV., Hughes, JC., Lamont, RA., Litherland, R., Jones, IR., Jones, RW., Knapp, M., Kotting, P., Martyr, A., Matthews, FE., Morris, RG., Quinn, C., Regan, J., Rusted, JM., van den Heuvel, EA., Victor, CR., Wu, YT. and Clare, L.. (2018) ‘Protocol for the IDEAL-2 longitudinal study: following the experiences of people with dementia and their primary carers to understand what contributes to living well with dementia and enhances active life.’, *BMC public health*, 18(1).
Usher-Smith, JA., **Silarova, B.**, Sharp, SJ., Mills, K. and Griffin, SJ.. (2018) ‘Effect of interventions incorporating personalised cancer risk information on intentions and behaviour: a systematic review and meta-analysis of randomised controlled trials.’, *BMJ open*, 8(1).
Usher-Smith, JA., **Silarova, B.**, Lophatananon, A., Duschinsky, R., Campbell, J., Warcaba, J. and Muir, K.. (2017) ‘Responses to provision of personalised cancer risk information: a qualitative interview study with members of the public.’, *BMC public health*, 17(1).
Dambha-Miller, H., **Silarova, B.**, Irving, G., Kinmonth, AL. and Griffin, SJ.. (2017) ‘Patients' views on interactions with practitioners for type 2 diabetes: a longitudinal qualitative study in primary care over 10 years.’, *The British journal of general practice : the journal of the Royal College of General Practitioners*, 68(666).
Usher-Smith, JA., Winther, LR., Shefer, GS., **Silarova, B.**, Payne, RA. and Griffin, SJ.. (2017) ‘Factors Associated With Engagement With a Web-Based Lifestyle Intervention Following Provision of Coronary Heart Disease Risk: Mixed Methods Study.’, *Journal of Medical Internet Research*, 19(10).
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Lophatananon, A., Usher-Smith, J., Campbell, J., Warcaba, J., **Silarova, B.**, Waters, EA., Colditz, GA. and Muir, KR.. (2017) ‘Development of a Cancer Risk Prediction Tool for Use in the UK Primary Care and Community Settings.’, *Cancer prevention research (Philadelphia, Pa.)*, 10(7).
Usher-Smith, JA., **Silarova, B.**, Ward, A., Youell, J., Muir, KR., Campbell, J. and Warcaba, J.. (2017) ‘Incorporating cancer risk information into general practice: a qualitative study using focus groups with health professionals.’, *The British journal of general practice : the journal of the Royal College of General Practitioners*, 67(656).
Shefer, G., **Silarova, B.**, Usher-Smith, J. and Griffin, S.. (2016) ‘The response to receiving phenotypic and genetic coronary heart disease risk scores and lifestyle advice - a qualitative study.’, *BMC public health*, 16(1).
Silarova, B., Nagyova, I., Rosenberger, J., van Dijk, JP. and Reijneveld, SA.. (2016) ‘Sense of coherence as a mediator between hostility and health-related quality of life among coronary heart disease patients.’, *Heart & lung : the journal of critical care*, 45(2).
Usher-Smith, JA., **Silarova, B.**, Schuit, E., Moons, KG. and Griffin, SJ.. (2015) ‘Impact of provision of cardiovascular disease risk estimates to healthcare professionals and patients: a systematic review.’, *BMJ open*, 5(10).
Silarova, B., Lucas, J., Butterworth, AS., Di Angelantonio, E., Girling, C., Lawrence, K., Mackintosh, S., Moore, C., Payne, RA., Sharp, SJ., Shefer, G., Tolkien, Z., Usher-Smith, J., Walker, M., Danesh, J. and Griffin, S.. (2015) ‘Information and Risk Modification Trial (INFORM): design of a randomised controlled trial of communicating different types of information about coronary heart disease risk, alongside lifestyle advice, to achieve change in health-related behaviour.’, *BMC public health*, 15.
Silarova, B., Giltay, EJ., Van Reedt Dortland, A., Van Rossum, EF., Hoencamp, E., Penninx, BW. and Spijker, AT.. (2015) ‘Metabolic syndrome in patients with bipolar disorder: comparison with major depressive disorder and non-psychiatric controls.’, *Journal of psychosomatic research*, 78(4).

Publication Record

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Publications
Silarova, B., Nagyova, I., van Dijk, JP., Rosenberger, J. and Reijneveld, SA.. (2013) ‘Anxiety and sense of coherence in Roma and non-Roma coronary heart disease patients.’, *Ethnicity & health*, 19(5).
Silarova, B., van Dijk, JP., Nagyova, I., Rosenberger, J. and Reijneveld, SA.. (2013) ‘Differences in health-related quality of life between Roma and non-Roma coronary heart disease patients: the role of hostility.’, *International Journal of Public Health*, 59(2).
Silarova, B., Nagyova, I., Rosenberger, J., Studencan, M., Ondusova, D., Reijneveld, SA. and van Dijk, JP.. (2013) ‘Sense of coherence as a predictor of health-related behaviours among patients with coronary heart disease.’, *European journal of cardiovascular nursing*, 13(4).
Silarova, B., Nagyova, I., Rosenberger, J., Studencan, M., Ondusova, D., Reijneveld, SA. and van Dijk, JP.. (2012) ‘Sense of coherence as an independent predictor of health-related quality of life among coronary heart disease patients.’, *Quality of life research : an international journal of quality of life aspects of treatment, care and rehabilitation*, 21(10).

Relevant Prizes, Awards and other Academic Distinctions

Please note the information for additional prizes or awards is automatically populated from information provided in the "Update CV" section of the "Manage My Details" page, which you can update by following this link https://aramis.nihr.ac.uk/MyAccount/UpdateCV.aspx

Date (mm/yyyy)	Source	Prize	Role of Applicant
01/02/2019	the Ludwig Boltzmann Gesellschaft, Austria	The Ideas Lab on Digital Health Meets Societal Needs: Selected for participation (30 participants were invited out of over 170 applications)	
01/02/2019	University of Exeter	the Springboard Women's Development Programme: Enables women to identify the clear, practical and realistic steps that they want to take to make a better world for themselves at work and home	
01/01/2019	The GW4 Alliance	GW4 Crucible 2019 – Digital Innovation programme: Selected for participation; aimed for aspiring future research leaders interested in focusing on the impact of digital innovation on the world	
01/01/2019	University of Exeter	Recognition for going Above & Beyond: I was recognised for helping a colleague who was not part of our research team with her research project	
01/06/2010	University of Rotterdam, The Netherlands	Nihes/ESP Fellowship: For the participation at the Erasmus Summer Programme 2010	
01/06/2010	Rijksuniversiteit Groningen, The Netherlands	Lifelong Learning Programme/Erasmus for Teaching Mobility: Erasmus for Teaching Mobility	
01/03/2010	European Commission	European Commission	

Relevant Prizes, Awards and other Academic Distinctions

Please note the information for additional prizes or awards is automatically populated from information provided in the "Update CV" section of the "Manage My Details" page, which you can update by following this link https://aramis.nihr.ac.uk/MyAccount/UpdateCV.aspx

Date (mm/yyyy)	Source	Prize	Role of Applicant
		Sponsorship: For attending 3rd European Public Health Conference - Integrated Public Health, Amsterdam, The Netherlands	
01/01/2010	University of Amsterdam, The Netherlands	SMART Fellowship: For participating at the Winter Course on Medicine and Human Rights	
01/06/2009	University of Trnava, Slovakia	The Deans´ Award for Excellent Study Results: Awarded to the top 5% students graduating that year	
01/08/2008	University of Trnava, Slovakia	Scholarship for Excellent Study Results	
01/08/2006	University of Trnava, Slovakia	Scholarship for Excellent Study Results	
01/08/2005	University of Trnava, Slovakia	Scholarship for Excellent Study Results	

ORCID iD 0000-0002-2473-2527

3. Applicant Research Background

Professional Background	Not a Registered Health or Care Professional

Profession (Not a Registered Health or Care Professional)
Psychologist

Please describe your research career to date

I am a psychologist who works in adult social care research with particular interest in family members and friends who support someone living with dementia (unpaid carers). There are two key messages from unpaid carers that inspired this Fellowship: *'carers are never asked what they needed/what they want'* and *'you can't pour from an empty jug*.' As such, I see this NIHR Advanced Fellowship as a beginning of my long-term commitment to help unpaid carers to have good overall health and wellbeing.

I have an MSc (research) in Clinical and Health Psychology, MSc in Psychology and PhD in Medical Sciences. I have ten years' experience in coordinating and successfully executing various research projects. I have acquired experience in all stages of the research process – from protocol development, ethics, conduct of studies of different designs (both quantitative and qualitative) through to dissemination of the results. Since 2014, I have built a research profile in Health, Social Care and Wellbeing. To date, I have co-authored 33 publications (12 as a first author and 11 as a second author) in peer-reviewed academic journals. According to ResearchGate, I have been cited 538 times, with an h-index 13. I worked as a researcher in the Netherlands, the USA and in England. I presented the research at international conferences focusing on health psychology, public health and adult social care. Importantly, I also ensure that outcomes of my research reach diverse audiences, including talks at carers' organisations and blogs focusing on unpaid carers.

My early postdoctoral career focused on developing and evaluating behavioural change interventions. For example, as a Career Development Fellow at University of Cambridge (2014-2016), I tried to understand the impact of communicating risk estimates on physicians' and patients' behaviour, wellbeing, and other relevant clinical outcomes. I used range of study designs including randomised controlled trial (RCT), qualitative interviews and focus groups as well as systematic reviews. Specifically, I was leading INFORM trial (Chief Investigator: Professor Simon Griffin), open labelled, parallel group RCT of cardiovascular disease risk communication. An integral part of INFORM was a personalised web-based lifestyle programme to prevent cardiovascular disease that I adapted for UK environment from American Heart to Health study. This has resulted in several papers with me being a first author on two key papers: the paper reporting the results from the RCT and protocol paper.

I expanded what I learned from cardiovascular disease prevention on dementia prevention during my time as a Research Fellow at University of Exeter (2017-2019). For example, as part of a wider project, I secured ESRC Impact Acceleration Account Business Boost funding (approx. £10,000) for the MOVE study to allow us to initiate collaboration with a mobile health application developer (based in Denmark) and to establish partnerships with representatives of key stakeholders (e.g. patients, health care professionals, commissioners and policy makers). During this time, I also became an Early Career Representative as part of International Research Network on Dementia Prevention. This resulted in an editorial (in Aging & Mental Health) co-authored with five international researchers highlighting that the career development of early- and mid-career researchers in dementia should be a global priority.

I started to be interested in a topic of wellbeing and health of unpaid carers who support someone living with dementia while working on projects that focused on understanding of unmet social care needs of people living with dementia and their unpaid carers (2019–to present). This has given me an incredible opportunity to build relationships with carers, health and adult social care professionals who support them and representatives from the voluntary sector across England. It has also enabled me to identify that there is a need for research that would specifically focus on physical and mental wellbeing of unpaid carers who support someone living with dementia. As a result of this, I successfully applied for an NIHR Three Schools: Dementia Research Programme Award (£32,166; Personal Career Award; July 2022-January 2023). This award was crucial in co-producing this Fellowship as it allowed me to build a research team with eight unpaid carers of people living with dementia (co-researchers). Between January and June 2023, we met online every month for two hours and worked together on what the focus of the Fellowship should be. In the second half of the year, we continued to meet monthly to discuss all work packages proposed in this Fellowship, dissemination activities and their role in the Fellowship if awarded. Alongside this Fellowship application, we also co-produced a blog[1] and we engaged with an illustrator

who captured our work on how unpaid carers of people living with dementia can address some of their concerns when it comes to their own health and wellbeing. In addition, our team also applied together with a local artist for SHAPE Involve and Engage grant from the British Academy (£8000, submitted in June 2023, unsuccessful) to enable us to hold a mixed-media workshop with unpaid carers of people living with dementia followed by an exhibition to increase awareness of what it means to be a carer for a person with dementia. We are also working together on academic paper and a podcast episode (for Dementia Researcher) to reflect on our experience of co-producing this Fellowship application.

To conclude, this NIHR Advanced Fellowship represents an incredible opportunity for a step change in my career trajectory – a crucial step of establishing myself as a future leader in dementia research. Namely, over the course of the Fellowship I would like to start to generate knowledge on how we can support unpaid carers of people living with dementia to look after their own health and wellbeing. Importantly, I would like for this knowledge to be co-produced together with unpaid carers. This will ensure that the research I will be leading on will be meaningful to unpaid carers, driven by their own needs. Ultimately as a researcher, I would like to make the needs of unpaid carers visible to academic community, present and future health and social care professionals as well as policy makers.

Title of research degree	PhD

Has the degree already been awarded?	Yes

Date of Award	27/01/2014

Has this application, or a similar application, previously been submitted to this or any other funding body?
No

Contextual Factors
Please use this question to detail any contextual factors you wish to make the Selection Committee aware of. NIHR wants to know about any circumstances so that they may take them into consideration during the assessment of your application. Contextual factors may include:

- Career breaks due to parental leave, or periods of illness.
- Reduced time spent undertaking research due to a disability or caring responsibilities. This could include any physical or mental difficulty that may have impacted your research career. These are situations that have a significant impact on your ability to undertake research.
- Reduced opportunities to career support e.g. mentorship, and limited opportunities to undertake prior research and training.
- Impact of the COVID-19 pandemic on your research career.

Please also use this section to detail any other factors that may have impacted your research career not listed in the examples provided. The impact on your career to date will be specific to your particular circumstances but could include such impacts as limited opportunities to obtain grant funding, or fewer publications. In general terms, mitigating factors should be significant, and relevant.

NIHR acknowledges that you may be reluctant, or uncomfortable disclosing relevant information that is sensitive. However, you should bear in mind that we are unable to take into account factors that you do not disclose. Please be assured that information provided by you is sensitive and will be treated confidentially and in line with General Data and Protection Regulations (GDPR).

There is one key mitigating factor that impacted my research career so far: the Covid-19 pandemic.

I joined University of Kent in September 2019. Six months later the first lockdown was enforced on 26th March 2020. This was followed by a series of lockdowns and restrictions until December 2021.

Since joining University of Kent, I have been working across several projects that include primary data collection in adult social care. While data collection in adult social care is challenging and time demanding, this became a particular issue during the COVID-19 pandemic. More specifically, adult social care was one of the most impacted sectors during this time. This meant for example that:

1. Data collection was completely halted and delayed. This was due to, e.g. being unable to obtain research

governance approvals. Many local authorities temporarily suspended their research governance approval process. Importantly local authorities and adult social care organisations were unable to support recruitment of participants.

2. Changes to data collection. As we experienced issues with data collection, we had to explore new recruitment pathways: e.g. data collection through NHS instead of adult social care organisations.

3. Data collection was time consuming: for example, to meet our target of recruiting 300 participants, I had to send 10000 individual emails to potentially eligible participants - this consumed the majority of my hours dedicated to the project. In addition, I was responsible for preparing study packages and sending them to participants.

The above are a few of the examples of how the Covid-19 pandemic impacted projects I was involved in. The key impact was that I spent much of my time on ensuring we would collect data on time and meet our recruitment targets. Hence, I had no time to dedicate to writing either research papers or applying for funding applications. Since 2022, I have been able to resume applying for research grants, leading on publications and leadership positions within my organisation in addition to my everyday tasks.

4. Plain English Summary of Research

Aim(s) of the research: The focus of this Fellowship is on family members and friends who support someone living with dementia (unpaid carers). Eight former and current unpaid carers highlighted that they would like to understand how they can increase their physical activity, improve their diet and manage their time better. Therefore, as a first step, we will prioritise which concern out of these three is the most important for unpaid carers living across England. As a second step, we will then design an intervention that will help unpaid carers to address the concern selected in the first step. As a last step, we will test whether the developed intervention is practical and acceptable for unpaid carers and for those who may be delivering it.

Background to the research: Across the UK there are 700,000 family members and friends who support a person living with dementia. In the academic literature they are referred to as carers, unpaid carers, family carers or supporters etc. but the family members and friends may not use any of these terms to describe themselves. As a result of their caring responsibilities, carers are often unable to do things that would support their own health and wellbeing. Unpaid carers themselves highlighted that '*It is exhausting to be a carer, it takes so much out of you*'. Currently, we know very little about how we can support unpaid carers to increase their physical activity, improve their diet and manage their time better.

Design and methods used: In this Fellowship we will follow a step by step guide called the Behaviour Change Wheel to understand how we can support unpaid carers to improve their health and wellbeing. In Stage 1 we will conduct a needs assessment (through reviewing existing evidence and collecting information among 100 unpaid carers living in England) to identify whether we should focus on e.g. helping carers to manage their time better (to have 'me time'). In Stage 2 we will use a range of methods to design the intervention itself. This will include a consensus exercise with 30 unpaid carers to define the exact focus of the intervention. We will then conduct five focus groups (3-5 people per group) with unpaid carers to gain further insight into how the intervention should look. We will then design the intervention itself and share it with five focus groups (3-5 people per group) including e.g. carers, health and social care professionals etc. to gather their feedback. In Stage 3 we will test whether the proposed intervention is practical and acceptable by the unpaid carers and the people who may deliver it.

Patient/service user, carer and public involvement: Unpaid carers played a central role in co-producing this Fellowship application. Over a period of twelve months we met on a monthly basis to decide together the focus of the Fellowship, how we are going to answer our research questions, disseminate results etc. We were a team of ten people - eight people self-identified as a former or current unpaid carer living across England and Scotland. They will continue to work on the Fellowship as co-researchers – this means they will be part of the core research team, they will jointly own the research and be responsible for it.

Dissemination: Unpaid carers highlighted that it is important that the results from this Fellowship reach a wide range of people including carers, health and social care professionals and that the results are shared throughout the Fellowship regularly. We will use a range of approaches, for example: Podcast/YouTube channel; articles; and working with an illustrator to capture some of the findings.

5. Scientific Abstract

Research question: What does a feasible and acceptable intervention targeting both physical activity and diet or time management of unpaid carers of people living with dementia look like?

Background: Across the UK there are 700,000 family members and friends of people living with dementia who are unpaid carers. While being a carer can be rewarding, caring is associated with a range of mental and/or physical health difficulties. Based on the evidence gap in the literature and working together with eight current and former unpaid carers, we identified a need to develop an intervention to improve unpaid carers' physical activity, diet and time management in order to support their health and wellbeing.

Aims and objectives: The ultimate goal of this Fellowship is to design and evaluate feasibility of a behaviour change intervention targeting both physical activity and diet or time management of unpaid carers of people living with dementia.

Methods: The study will use mixed methods across three stages of work. Eight unpaid carers co-produced the suggested research and will be part of the core research team throughout the Fellowship. In Stage 1 (needs assessment) we will conduct a survey (online or paper) with 100 unpaid carers of people living with dementia across England to identify whether the focus of the intervention should be on physical activity and diet or time management. This decision will also be supported by a systematic review to understand where the highest need for an intervention development is. In Stage 2 we will develop the intervention itself focusing on key concern(s) as identified in Stage 1, e.g. diet and physical activity. We will use a range of methods as recommended by the Behaviour Change Wheel. First, we will conduct an online Delphi consensus exercise with 30 unpaid carers to define target behaviours (e.g. healthy snacking; healthy cooking etc.). We will then conduct five focus groups (3-5 people per group) with unpaid carers to define the exact focus of the intervention including what needs to change in the person and/or the environment in order to e.g. stop snacking on high-fat/salt/sugar snacks. Once we understand what needs to change, the research team with the support of other experts will design the intervention itself and conduct five focus groups (3-5 people per group) with e.g. unpaid carers, other family members, health and social care professionals etc. to gather feedback. In Stage 3 we will conduct a feasibility study to test whether the proposed intervention is feasible and acceptable for unpaid carers and those who may deliver it.

Timelines for delivery: Stage 1: 1-12 months; Stage 2: 13-29 months; Stage 3: 30-44 months.

Anticipated impact and dissemination: A functional, feasible, and acceptable intervention targeting physical activity, diet or time management of unpaid carers of people living with dementia across England. For dissemination of results, we will use a range of approaches, for example: Podcast/YouTube channel; webinars; articles for the Alzheimer's & Dementia Journal or the Journal of Dementia Care; working with an illustrator to capture some of the findings etc.

6. Detailed Research Plan

What is the problem being addressed?

Dementia is fast becoming the defining health and social care issue of our time. It is estimated that there are currently 944,000 people with dementia in the UK and this is likely to increase with an ageing population[2].

As dementia progresses, many people find it increasingly difficult to look after themselves and their homes and may need help with daily activities[3]. Most of this help is provided by family, close friends or neighbours (also referred to as unpaid, informal or family carers). In this Fellowship, I use the term 'unpaid carers', a term preferred by eight former and current unpaid carers of people living with dementia (co-researchers) who co-produced this application.

Unpaid carers support individuals with dementia in a range of ways, including personal care, cooking, finances, emotional support and arranging support from services[4]. It is estimated that unpaid carers spend 1.1 billion hours each year supporting people with dementia[5]. As highlighted by Fiona Carragher from the Alzheimer's Society: '*Unpaid carers of people with dementia, are part of the backbone of the care sector, but these unsung heroes are being pushed to breaking point*'[6]. Importantly unpaid carers of people living with dementia highlighted while working on this application that '*Carers are overwhelmed as it is expected from them to do it all*' and '*It is time to recognise how the caring impacts family members and friends of those living with dementia*'.

Between January 2023 and January 2024, I met monthly with eight unpaid former or current carers (co-researchers) of people living with dementia who live across Scotland and England. They identified 22 health and wellbeing concerns experienced by carers of people living with dementia, the most important ones being: 1.Exercise, 2. Diet, 3. Coping with stress, 4. Coping with terminal illness, 5. Coping with emotions, 6. Getting help, 7. Knowledge and 8. Managing their time better[1].

At the moment, very little is known about **physical activity, time management and diet of unpaid carers of people with dementia** when compared to other concerns (copying with emotions and stress[7]; knowledge[8]; copying with terminal illness/grief[9] and getting help e.g. from adult social care services[10]).

Therefore, the focus of this Fellowship is **on unpaid carers of people living with dementia in England and how we can support them to improve both their physical activity and diet or to manage their time better.**

Why is this research important in terms of improving the health and/or wellbeing of the public and/or to patients/service users, carers and health and care services?

The total costs of dementia in the UK in 2019 amounted to £34.7 billion. **Unpaid care account for 40% (£13.9 billion)** of the total costs[11]. Without unpaid carers it would be financially impossible for health and adult social care to provide the huge amount of unpaid care (e.g. up to 100 hours per week during pandemic[12]) provided by carers[11].

While caring for someone living with dementia can be rewarding[13], unpaid carers of people living with dementia experience a range of mental and physical health issues(e.g.[14-16]) including stress, depression, anxiety, grief and physical and emotional burden. Unpaid carers have an increased mortality[17] and worse physical and mental health outcomes especially among women, those looking after a spouse, and those providing very high levels of care[18]. An Alzheimer's Society survey in 2020 found that two-thirds of unpaid carers of people living with dementia '*desperately needed more support to protect their wellbeing*'[6].

Review of existing evidence: how does the existing literature support this proposal?

Unpaid carers of people living with dementia experience a range of mental and physical health issues (e.g. (e.g.[14-16]). Very little is known about **physical activity, time management and diet of unpaid carers of people with dementia** when compared to other concerns (e.g. copying with emotions and stress[7]; knowledge [8]; copying with terminal illness/grief[9] an getting help e.g. from adult social care services[10]).

No studies have explored how unpaid carers of people living with dementia can improve their time management. The need to manage time better was identified by co-researchers themselves. Examples included: '*I never get any time for myself*'; '*I have no time to do shopping today*' and '*I am so bad at managing my time'*.

No studies have explored how unpaid carers of people living with dementia can improve their diet. Again, the need to improve diet was identified by co-researchers. Examples included: neglecting own meals while focusing on

person with dementia; unhealthy snacking; poor quality ready meals; and binge eating.

A systematic review of physical activity interventions for unpaid carers of people with dementia found that studies were too methodologically weak to allow robust conclusions[19]. Methodological issues of the included studies were also reported in another systematic review of exercise for caregiver–care recipient dyads[20].

What is the research question/aims and objectives?

The ultimate goal of this Fellowship is to design and evaluate the feasibility of a behaviour change intervention targeting physical activity, diet or time management of unpaid carers of people living with dementia. Throughout the Fellowship I will answer the following research question: What does a feasible and acceptable intervention targeting both physical activity and diet or time management of unpaid carers of people living with dementia in England look like?

Project Plan

The proposed work is informed by Medical Research Council guidance for developing and evaluating complex interventions[21]; the Behaviour Change Wheel[22]; and Co-production.

The study will use mixed methods across three stages of work (Figure 1. Research plan):

1. Stage 1 (needs assessment; 1-12 months)
2. Stage 2 (intervention development; 13-29 moths)
3. Stage 3 (feasibility study; 30-44 months).

Equality, diversity and inclusion (EDI): information will be collected and results will be summarised where possible based on unpaid carer's ethnicity/race, age, gender and sexual orientation. We will ensure that we will recruit participants from a range of backgrounds including recruiting participants from specific areas to sample different ethnicities, socioeconomic backgrounds and levels of access to health and dementia care services. We will also engage with organisations that specifically support LGBTQ+ communities. I am also a member of Dementia Community EDI specialty interest group and will draw on the members' expertise.

Examples of the organisations that we will engage with are: Opening Doors; Switchboard Dementia; LGBTQ+ Dementia Advisory Group team; The Ethnic Minorities in Canterbury (EMIC); Diversity Resource International; the West Sussex Alzheimer's Society (link established with chair of the Dementia Strategy Minorities Sub-group), Carers Support West Sussex (link with Bi-lingual Dementia Navigator); and Meri Yaadain CiC (supporting ethnic communities). I will also draw on the links of the Clinical Research Network lead (Kent, Surrey and Sussex), namely the Research Engagement Network that have connections with Somalis and South Sudanese communities.

The length of each focus group will be up to one hour and times will be provided outside typical working hours. Both online and in person focus groups will be offered. We will contribute to carers' replacement costs so participants can take part in research while a professional is supporting a person with dementia. If people do not wish to participate in a focus group, e.g. due to a language barrier, an option will be provided to have an individual interview with an interpreter present. All documents will be translated into three additional languages.

Stage 1: Needs assessment; 1-12 months: survey and systematic review

While there is no agreement on how many concerns an intervention should target, the intervention itself will only focus on one of these concerns: 1. both physical activity and diet or 2. time management (e.g. prioritising time for yourself). As in current practice it would be unusual in public health to address physical activity and diet separately, if selected the plan is that the intervention itself will target both. However, as part of the survey and systematic review we will explore whether indeed the intervention should target both diet and physical activity or whether the focus should be on one only.

Stage 1a: Needs assessment; 1-12 months: survey

Aim: to identify the top health and wellbeing concern of unpaid carers of people living with dementia. In addition, we will aim to gather input on the key barriers and enablers to support unpaid carers to tackle these concerns; mode of delivery of the intervention; and who should deliver it. This information will be then used in Stage 2 to inform intervention development.

Study design: a self-administered questionnaire for unpaid carers in England. Participants will be able to choose between a postal or online version (using Qualtrics XM Platform™). It is important to highlight that the James Lind

Alliance Priority Setting Partnership[23] was not considered as appropriate method for this study as we have already done some of the ground work. For example, the proposed three health and wellbeing concerns were suggested by eight unpaid carers themselves. However, as highlighted by co-researchers: '*Everyone is different and it is important to take time to evaluate people's needs*'. Therefore, there is a need to conduct a survey among wider group of unpaid carers to identify the top health and wellbeing concern.

Survey: all study documents including survey will be developed together with eight co-researchers. The survey will include questions about:

1. The Caregiving situation: former vs current carer, whether the carer and person with dementia live in the same household, relationship to a person living with dementia, estimated hours of care per week, any additional support received from other family members and friends, health care and adult social care services etc.
2. Sociodemographic characteristics: age; gender; ethnicity; region; urban vs rural area; socioeconomic status; English as a second language.
3. Carer quality of life of someone with dementia: a condition-specific measure of the quality of carers of people with dementia[24].
4. Mode of data collection: paper or online.
5. Top health and wellbeing concern: able to choose from the three health and wellbeing concerns suggested by co-researchers as described earlier. Participants will have an opportunity to suggest other health and wellbeing concerns and will be provided also with a free text box to explain their decision.
6. To inform the Intervention development: questions around mode of delivery of the intervention, who should deliver it and what are the key barriers and enablers to support unpaid carers to tackle these concerns (questions informed by both COM-B model[22] and the Theoretical Domains Framework (TDF)[25].

Participants and setting: unpaid carers of people living with dementia across England. Both former (within five years[26]) and current carers will be eligible to take part, if they provide or provided unpaid support to a relative, partner/spouse or friend with dementia. They will have to be at least 18 years old. While this survey is not James Lind Alliance Priority Setting Partnership per se, I used their recommendation on sample size to determine how many participants need to be recruited for this survey. At present, there is no recommended maximum or minimum number of responses. Survey responses from previous priority settings have varied from 100s to 1,000s[23]. As such we will aim to recruit at least 100 participants (maximum 500). We will distribute the survey through multiple channels: GP practices, memory services, community pharmacies, social media (e.g., Twitter), previous studies, online dementia research websites (e.g. Join Dementia Research (JDR)), carer organisations and dementia services. The recruitment will be opened up to four months.

Analysis: descriptive analysis (e.g. frequency, median). We will also explore whether there are any differences in the selected top health and wellbeing concerns between e.g. former and current carers; carers from different socioeconomic backgrounds etc. If we find differences, this will be discussed with the Steering Committee (SC) and their advice will be sought on how to deal with this.

Outcome: top health and wellbeing concern of unpaid carers of people living with dementia. Information about mode of delivery for the intervention, who should deliver it and any key barriers and facilitators. In addition, we will establish the suitability of recruitment pathways and whether any changes to recruitment have to be introduced for the Stage 2.

Stage 1b: Needs assessment; 1-12 months: systematic review and meta-analysis

Alongside the survey (described in Stage 1a), we will conduct a systematic review and meta-analysis.

Research question:

1. Are there effective interventions to improve: physical activity; diet; and time management among unpaid carers of people living with dementia and other conditions?

Study design: systematic review and meta-analysis. Protocol for this review will be registered a priori in the PROSPERO database. The reporting will follow the Preferred Reporting Items for Systematic reviews and Meta-Analyses (PRISMA) guideline[27].

Eligibility criteria, information sources and search strategy: only studies published in English in peer-reviewed journals will be included. We will only include randomised trials. As highlighted earlier there are limited studies among unpaid carers of people living with dementia focusing on diet, physical activity and time management.

Therefore, I will broaden the inclusion criteria on other carers that may have similar experiences – this will be guided by discussion with co-researchers, SC and by the number of studies identified. The outcomes of interest will be levels of physical activity; diet; and time management. The search strategy will be developed in consultation with university librarian and mentors. The titles, abstracts and full texts will be screened by at least two reviewers.

Risk of bias: we will use Critical Appraisal Checklists[28] to assess the risk of bias in included studies.

Data synthesis and statistical analysis: the detailed statistical plan will be reviewed by a statistician. Where possible I will combine data from different studies using random effects meta-analysis.

Outcome: comprehensive list of interventions including whether they work, in which setting they were tested and with whom.

At the end of Stage 1, I will organise an SC meeting. The aim of this meeting will be to review the findings from Stage 1 and decide which health and wellbeing concern will be taken forward to Stage 2. As suggested by Medical Research Council guidance[21] we will consider the following criteria when making this decision: priority; opportunity to intervene; ability to change; size of potential benefit; stakeholder desirability for an intervention etc. Implementation challenges will be also considered.

Stage 2: **Intervention development; 13-29 moths**

Stage 2 will focus on development of an intervention for health and wellbeing concern as identified by the end of Stage 1. The Intervention development will be informed by the Behaviour Change Wheel[22]. It includes following steps: a) Understanding the behaviour; b) Understanding what needs to change; c) Identifying intervention options; and d) Identifying content and implementation options.

Stage 2a: **Understanding the behaviour; 16-21 months: Delphi consensus exercise**

Aim: to understand the behaviour that will be the target of the intervention.

Study design: we will conduct a Delphi consensus exercise as recommended by authors of Behaviour Change Wheel with a series of rounds where stakeholders produce a list of target behaviours ('What behaviour needs to change?') and actors ('Who is involved?'). For example, as a result of Stage 1, we may agree that the focus of the intervention should be improved diet. However, improved diet is not a behavioural target. Behavioural targets for improved diet can include for example: buying groceries on the list; batch cooking and freezing portions ready to eat; carrying healthy snacks etc. For 'who is involved' examples can include, e.g. unpaid carer; meal service provider; friends etc. The aim of the Delphi exercise will be to generate all the target behaviours and actors (e.g. unpaid carers; meal providers etc.).

Delphi consensus exercise: We will conduct a 2-round online modified Delphi consensus exercise using Qualtrics XM Platform™. The Delphi consensus exercise is designed to elicit a 'consensus of opinion' from a group of experts. Participants remain anonymous and provide feedback as part of the iterative process[29]. The aim of the Delphi consensus exercise will be to develop a consensus among unpaid carers of people living with dementia on a list of target behaviours and actors. It will include background questions about the unpaid carer and their sociodemographic characteristics (similar to Stage 1a). The initial list of questions, 'What behaviour needs to change?' 'Who is involved?' 'What makes it more or less likely to succeed?', for the expert panel to consider were already generated by eight co-researchers involved in this application. Examples for e.g. diet can be found here[1]. In addition, we will include suggestions as collected in Stage 1a (survey).

Participants and settings: unpaid carers of people living with dementia across England. Both former (within five years[26]) and current carers will be eligible to take part, if they provide or provided unpaid support to a person with dementia. They will have to be at least 18 years old. It is important to note that there is no agreement on the minimum panel size required for content validity in Delphi studies. Recommendations vary from 5-30 participants[30, 31]. Given the diversity of experiences in carers of people living with dementia, we will aim to recruit up to 30 participants. We will include and build on our recruitment strategies as suggested in Stage 1a. Participants taking part in Stage 1a will be also asked whether they would agree to be contacted in future about research studies. The recruitment and data collection will be opened to four months. I acknowledge that those who do not have access to internet/do not wish to engage in Delphi online exercise will be excluded from this study.

Analysis: descriptive statistics (e.g. median and frequencies) for representing the group opinion.

Outcome: core behaviours will be identified. We will understand: 'What behaviour needs to change?' 'Who is

involved?' 'What makes it more or less likely to succeed?'

Once the Delphi exercise is finalised I will organise an SC meeting. Building on the results from the Delphi consensus exercise we will make a final decision which target behaviours the intervention should address using the criteria of impact, likelihood of change, positive or negative spill-over to other behaviours and ease of measurement of outcomes. We will ensure that this decision is culturally, socially and context driven and acceptable. The target behaviours will be described in terms who needs to perform the behaviour, what the persons need to do differently, when, where, how, and with whom will they do it.

Stage 2b: Specify what needs to change; 19 – 24 months: focus groups

The COM-B model suggests that changing the incidence of any behaviour of an individual, group or population involves changing one or more of the following: capability (physical/psychological), opportunity (physical/social), and motivation (reflective/automatic) relating either to the behaviour itself or behaviours that compete with or support it[22]. These high-level constructs can be understood further using the Theoretical Domains Framework (TDF)[25]. The more accurate this analysis of the target behaviour, the more likely it is that the intervention will change the behaviour in the desired direction[22].

Aim: to identify what needs to change in the person and/or the environment in order to achieve the desired change in behaviour(s) as specified at the end of Stage 2a.

Study design: we will use five focus groups. The co-researchers agreed that focus groups are a good way to answer our research questions. It was highlighted that the focus groups should be small, e.g. 3-5 people. Both online and in-person options will be provided to conduct the focus groups. We will build on recruitment strategies as established in previous Stages. Focus groups questions will be informed by both COM-B model[22] and TDF[25]. We will also ask what could help participants overcome challenges/barriers in changing their behaviour mentioned during the focus groups. In addition, we will also collect background information (e.g. EDI characteristics) and ask about caring situation.

Participants and settings: unpaid carers of people living with dementia across England. Both former (within five years[26]) and current carers will be eligible to take part, if they provided or provided unpaid support to a person with dementia. They will have to be at least 18 years old. We will use similar recruitment strategies as in the previous Stage. We will also approach participants who took part in the previous Stage if they agreed to be contacted.

Analysis: two researchers will independently code the transcripts. The COM-B model and TDF will be used as a combined deductive framework for the analysis. The interview data will be managed using NVivo. SPSS will be used for descriptive data analysis of participants' characteristics.

Outcome: understanding what influences behaviour through a behavioural diagnosis using the TDF and COM-B.

Stages 2c) Identifying intervention options; 25-30 months: research group discussions

Aim: to identify relevant intervention functions and policy categories following the COM-B and TDF analyses.

As a first step we will present findings from the COM-B and TDF analyses (Stage 2b) at SC meeting. We will discuss which themes should be prioritised, e.g. using the criteria of frequency (e.g. how many times does psychological capability feature as an influence in data we collected); elaboration (number of themes and sub-themes within each domain) and expressed importance (e.g. quotes from study participants expressing importance). Once we agree on key barriers and facilitators, these will be mapped against nine intervention options and seven policy categories as outlined in Behaviour Change Wheel[22]. The identified intervention options and policy categories will be assessed with co-researchers and with an SC against APEASE criteria[22, 32]: acceptability, practicality, effectiveness, affordability, side-effects/safety and equity.

Stage 2d) Identifying content and implementation options; 25-29 months: research group discussions

Aim: to identify which techniques will deliver the intervention options and how the intervention will be put into practice.

Behaviour change techniques are the active ingredients within the intervention, designed to change the behaviour (e.g. goal setting, behavioural substitution, feedback on behaviour etc.). The behaviour change techniques will be selected using Behaviour Change Techniques Taxonomy App as well as existing literature on what behaviour change techniques are best for changing exercise/diet. The identified behaviour change techniques will be assessed with co-researchers and with an SC against APEASE criteria[28, 32]. We will also identify behaviour change techniques mentioned within the focus groups in Stage 2b. The discussion with co-researchers, SC and the findings from focus groups will then inform the development of intervention. We will also discuss the most appropriate style and mode of delivery of each technique (e.g., professional conversation, peer support buddies, leaflets, website, mobile apps etc.)[22]. Decisions will also be made around who should deliver the intervention, and the intensity, frequency and duration of the intervention.

Stage 2e) Gathering feedback on intervention; 28-33 months: focus groups

We will share the intervention with a range of stakeholders living across England. The final decision who should be involved in these groups will depend on how the intervention will look and who may be delivering it. With co-researchers, we have currently identified the following stakeholder groups: unpaid carers; other family members and friends; health care professionals; people working in adult social care and social work. We will conduct up to five focus groups with 3-5 people per group. Both online and in-person options will be provided to conduct the focus groups. We will build on previous recruitment strategies.

Analysis: focus groups will be audio-recorded using encrypted digital voice recorders and transcribed. The content of the focus groups will be analysed using content analysis. The intervention will be adapted as needed.

Outcome of Stages 2c, 2d and 2e: the intervention will be developed.

Stage 3: Feasibility study; 30-44 months

Aim: to test whether the proposed intervention is feasible and acceptable for unpaid carers and those who may be delivering it.

Study design: feasibility study conducted in England. A quantitative (e.g. surveys) and qualitative (e.g. semi-structured interviews) mixed-methods design will be used to assess predefined progression criteria that relate to the evaluation design (e.g. reducing uncertainty around recruitment, data collection, retention, outcomes, and analysis) and the intervention itself (e.g. around optimal content and delivery, acceptability, adherence, or capacity of providers to deliver the intervention).

Dissemination, Outputs and anticipated Impact

We will work closely with SC and co-researchers to develop and deliver a dissemination strategy that is impactful and appropriate for different target audiences. Results from each Stage/study will be shared with participants and participating health care and social care organisations. We will also disseminate results through SC and co-researchers will use their established links, e.g. Mobilise, Facebook groups, their own carers' groups, Dementia Friendly East Lothian, Local Advocacy groups etc. Co-researchers suggested the following outputs to disseminate the findings: blog; leaflets; infographic; webinar; an article for Alzheimer's Society journal or the UK Dementia Journal. We will also write peer-reviewed papers and co-present the results at conferences.

Importantly after each Stage we will revisit and adapt our dissemination strategy. We will also adapt the dissemination strategy based on the training obtained through this Fellowship.

I will also set up a project website/page aimed at academics and media outlets. We will also produce a 12-minute UK based documentary with KMTV (TV made for Kent). The primary aim of this output is to disseminate the research findings and to engage policy makers and media stakeholders. It will be transmitted on the Local Television Network Channel 7 through KMTV, together with the coordination of associated press and publicity campaigns undertaken with partner organisations.

We anticipate the following impact:

- By the end of this Fellowship a feasible and acceptable intervention targeting physical activity, diet or time

management of unpaid carers of people living with dementia will be developed. Following this, the plan will be to apply for funding to evaluate whether the proposed intervention works.
- Increased awareness of health and wellbeing concerns experienced by unpaid carers of people living with dementia among health and social care professionals, policy makers and wider public.
- Other target behaviours for possible future interventions will be identified.
- This Fellowship addresses health and wellbeing concerns as identified by unpaid carers. This Fellowship has a potential to improve how the unpaid carers are supported in terms of their own health and wellbeing concerns by identifying who needs to be involved in supporting them, as well as any barriers and facilitators to behaviour change.
- This Fellowship has a strong focus on EDI. Therefore, we will be able to share findings with range of unpaid carers considering their gender, sexual orientation, ethnic background, socioeconomic status etc.

Project Management

I will be responsible for project co-ordination and management, budget management, ensuring effective communications within the team, overseeing our publication and dissemination strategy.

Within this Fellowship application I have requested funding for support staff (research associate) to the equivalent of 50% FTE for a period of 44 months. We will also identify together any training needs (£3000 was requested to support them). They will be part of project management group and attend SC meetings.

Project Management Group (PMG) will meet monthly to ensure the project is on track and discuss project related issues. The group will include myself, support staff and eight co-researchers. Each member of the PMG will have an opportunity to chair these meetings. In addition to attending monthly meetings, co-researchers expressed interest in conducting the research itself including conducting focus groups, reviewing all study documents and supporting dissemination activities. We agreed with co-researchers that on top of monthly meetings they may contribute up to 10 hours per month to the research delivery. The type of activities and amount of commitment will be decided with each co-researcher depending on their availability at that time as their situation and health can fluctuate. Appropriate training will be provided and was included in the budget.

The SC will meet online at key stages of the project. The support staff, co-researchers and mentors will also attend all SC meetings. SC will include academics with relevant expertise, health and social care professionals, representatives from independent charities and commissioning officers.

The Brighton and Sussex Clinical Trials Unit (CTU): will support me in three areas: data management, study management and statistical advice.

Ethics: before the start of the study, a favourable opinion will be sought from the Health Research Authority and/or University of Kent. When needed Research Governance approvals will be sought from participating local authorities and ADASS (Directors of Adult Social Care).

Success Criteria

The Fellowship will be a success if I develop feasible and acceptable intervention targeting physical activity and diet or time management of unpaid carers of people living with dementia. The ongoing success criteria would be to deliver each stage on time, recruit as intended and deliver outputs as planned.

Barriers/risks include the following:
- Failure to start on time due to delays in contract, ethical approval or recruitment. Risk low, allowing enough time between outcome of the competition and start of the Fellowship and allowing enough time for securing approvals.
- Failure to recruit members to SC: Risk low, most have already agreed.
- Failure to recruit: Risk low, I have established relationships in the field and the focus of this Fellowship is important as seen by unpaid carers, health care professionals and people working in adult social care. Importantly I have previous experience with recruiting unpaid carers of people with dementia through health care and social care. I will also draw on links of eight co-researchers. I will also be supported through CRN.
- Failure to deliver each element on time. Risk: medium. I have allowed 3.5 years to deliver this Fellowship. I also requested a support from research associate. Importantly I will be also supported by eight co-researchers (e.g. 10 hours per month each to support e.g. recruitment, focus groups etc.), CTU, mentors

and by University of Kent.

7. Patient & Public Involvement

Please describe how patients/service users, carers and the public have been involved in developing this proposal

The focus of this Fellowship is on unpaid carers of someone living with dementia. Therefore, unpaid carers played a central role in co-producing this Fellowship application. Over the period of twelve months (January 2023-January 2024) we met together online on a monthly basis for two hours. Throughout these meetings we got to know each other, we agreed how we wanted to work together, identified health and wellbeing concerns for unpaid carers, what the wellbeing and health-related goals carers would like to work towards etc. We also spoke about any barriers and facilitators when it comes to supporting carers to meet their own health and wellbeing goals. For example, it was highlighted that: '*It is important that professionals give advice that is purposeful and meaningful and would allow carers to act on it. Often advice around lifestyle changes is not applicable to carers*.' In addition, unpaid carers expressed that: '*There is no size fits all, the intervention has to be tailored to individual circumstances, it is important to speak to people and to listen to them*.' In our meetings, we also spoke together about each stage. We specifically focused on what methods are the most appropriate way to collect the information, who should be the target population, how we would like to disseminate the results etc. They also provided feedback on the application itself to ensure that their voices run as a golden thread through this application.

We were a team of ten people. Eight people self-identified as a former or current unpaid carer living across England and Scotland. Seven co-researchers will continue to work on the Fellowship as co-researchers – this means they will be part of the core research team; they will own the research and be responsible for it. As one person passed away we will recruit a new co-researcher for this Fellowship. The co-researchers themselves represent a range of perspectives in terms of their experiences as carers. Importantly the group also included individuals with diverse characteristics: e.g. having English as a second language; being black, belonging to LGBTQ+ community and coming from range of socioeconomic backgrounds.

Please describe the ways in which patients/service users, carers and the public will be actively involved in the proposed research, including any training and support provided

Seven unpaid carers will continue to work on the Fellowship as co-researchers. As one person passed away we will recruit a new co-researcher for this Fellowship. We will specifically recruit someone from a minority background to further diversify the voices represented in our group.

The co-researchers will participate in monthly two hour online project management meetings. The frequency, format and length of meetings were suggested by co-researchers themselves and this worked well for us in the last twelve months. If the person cannot attend the meeting, they will be offered a replacement meeting – a practice that we established while working together on this application. Each person will receive ahead of the meeting study packages (either printed or electronic version) including any materials that will be discussed during the meeting. Co-researchers will have an opportunity to chair these meetings and facilitate any breakout groups during the meetings. They will be paid for their attendance at the meetings including contribution to their internet costs. Importantly to allow them to contribute we also budgeted for carer replacement costs: some carers of people living with dementia may require having someone in place instead of them to look after a person living with dementia while working on the project. Co-researchers will also attend all SC meetings.

In addition to attending monthly meetings, co-researchers expressed interest in conducting the research itself including conducting focus groups, reviewing all study documents, and supporting dissemination activities. We agreed with co-researchers that on top of monthly meetings they may contribute up to 10 hours per month to the research delivery. The type of activities and amount of commitment will be decided with each co-researcher depending on their availability at that time as their situation and health can fluctuate.

Co-researchers will be provided with training to supplement and enhance their existing skills so they are able to carry out research activities related to the project. Each of them will also be able to attend two conferences throughout the Fellowship.

If it is considered not appropriate and meaningful to actively involve patients/service users, carers and the public in your proposed research, please justify why

Not applicable

8. Training & Development and Research Support

Proposed training and development programme

1. **Training focused on behaviour change interventions:** The proposed research is informed by Behaviour Change Wheel. In 2022-2023 I attended two courses in Behaviour Change Wheel: Behaviour Change Interventions: Introductory Principles and Practice (introductory) and Applied Principles and Practice of Behaviour Change (intermediate) to support the development of this application. During the Fellowship I would like to gain more in depth-knowledge of Behaviour Change Wheel and therefore I propose to attend two Advanced Modules in Behaviour Change: Principles and Practice of Behavioural Systems Mapping and the Behaviour Change Intervention Ontology (BCIO): How to use the BCIO for different research and applied purposes. I plan to attend these courses during the first year of the Fellowship. These courses are delivered by the developers of the model based at Centre for Behaviour Change, UCL. In addition to gaining a specific knowledge, attending these courses will also allow me to connect to other researchers interested in this specific approach. Importantly, Behaviour Change Wheel represents one of the approaches to behaviour change. There are other approaches to developing interventions e.g. Intervention Mapping which provides a framework for developing theory-based and evidence-based interventions applied to health promotion and disease prevention. To be able, in the future, to support other researchers aiming to improve the wellbeing and health of unpaid carers of people with dementia, I also propose to attend a 5-day summer course on Intervention Mapping: Designing Theory-based and Evidence-based Programs at Maastricht University in the Netherlands. Being trained in two key approaches to behaviour change intervention development will enable me to work beyond this Fellowship on a range of projects and connect with people who apply these approaches in their work. I plan to attend this course in the second year of the Fellowship.
2. **Training in Co-production**: as a researcher I am committed to work in a co-produced way together with unpaid carers of people living with dementia, other academics, health and social care professionals. While co-producing this application over the period of twelve months together with unpaid carers I have identified two key areas where I need additional training: 1. working with conflict and confrontation in teams; and 2. facilitation skills. As such I plan to attend three courses delivered by Gestalt Centre in London during the first year of Fellowship: Group Facilitation: Key concepts, skills and personal development (one day in person workshop); Working with Conflict and Confrontation in Groups and Teams (two days in person workshop); and Group Facilitation Training Intensive (nine days in total spread over four months).
3. **Equality, Diversity and Inclusion (EDI)**: As a researcher, I am aware that the research I conduct mainly reflects experiences of those who self-identify as e.g. white, British, heterosexual and from middle class. As such in this Fellowship, I intend to ensure that we will take proactive steps to recruit people from diverse backgrounds – this includes co-researchers, members of SC and research participants. While as an EDI representative at University of Kent I have opportunities to attend a range of training sessions, in this Fellowship I would like to specifically attend training sessions related to EDI issues when conducting research. Therefore, I plan to attend following training throughout the Fellowship: a) Improving the inclusion of under-served groups in health research (online webinar by INVOLVE); and b) An Ethical Perspective on Implementing Equal Opportunities in PPI (online webinar). Importantly as part of the NIHR Applied Research Collaboration I am member of a working group of researchers interested in the inclusion of individuals living with dementia and their carers in research.
4. **Impact and dissemination training:** I aim to become a researcher who has an influence beyond academia. Specifically, I would like to make the needs of unpaid carers visible to present and future health and social care professionals as well as policy makers. I plan to attend the following courses: a) Impact Training for Researchers (online course provided by Fast Track Impact over 5 weeks; first year of the Fellowship) and Compelling Communication Skills (6 week course delivered by University of Cambridge; second year of the Fellowship). In addition, I will apply for the Royal Society Pairing Scheme. Each year 30 research scientists are paired with UK parliamentarians and civil servants. They learn about each other's work by spending time together in Westminster and the researcher's institutions. Those taking part gain an insight into how research findings can help inform policy making and come away with a better understanding of how they can get involved.
5. **Leadership training:** I would describe a good leader in three words: kind; adaptive and self-aware. I had an opportunity to attend the NIHR Virtual Leadership Programme for social care researchers which was a scaled down version of NIHR Future-Focused Leadership Programme. As the content of that course reflected my own values, I would like to attend the NIHR Future-Focused Leadership Programme as part of this Fellowship.
6. **Software skills:** I would like to become an expert on two software packages I regularly use in my research and will be also central to delivery of this Fellowship: NVivo and Qualtrics. As such I suggest attending two courses: NVivo Expert Certification: this will enable me to be an in-house NVivo expert and advisor for other researchers across University of Kent. I will also do Qualtrics online courses, again allowing me to raise my profile in the research community.

7. **Specific skills and knowledge needed for the next stage of the project (after Fellowship):**
- Implementation Science Training: designed to build capacity and prepare for implementation in particular settings; third year of the Fellowship.
- Economic Evaluation Online: An introductory course; third year of the Fellowship.

8. **Conferences:** Conferences are an incredible opportunity to present one's own research, to learn about up to date evidence and make important connections. As such I plan to attend at least one conference between years 2-3 of the Fellowship. I will focus on conferences in three areas: dementia; health psychology/behaviour change; and ageing.

RESEARCH SUPPORT / SUPERVISOR 1

Name	Dr Rosie Ashworth

Institution	NHS Scotland Neuroprogressive and Dementia Network

Position	The Lead for the Patient and Public Involvement group

Description of support provided
Rosie leads a group known as 'Partners in Research' that includes people living with dementia and their unpaid carers. They work together on a range of activities including book writing, developing research proposals, reviewing research materials and raising awareness. Rosie is also a Research Fellow at the University of Edinburgh with a special interest in co-produced research. Rosie played a central role in development of this Fellowship application as she provided me with advice on co-production in the last twelve months. She also helped me to co-facilitate the majority of the meetings with unpaid carers in the last twelve months as we worked together on this application. We have worked together on co-producing a blog, academic paper, podcast episode and another funding application. Rosie will continue to support me regularly throughout the Fellowship. She will provide advice on any issues with co-production, co-facilitate meetings with unpaid carers when needed and she will be part of SC. We will have monthly online meetings to discuss the progress of this Fellowship. She will also be involved in key research outputs from this Fellowship. Importantly, Rosie has established links with other researchers in dementia working in a co-produced way and will help me to network with other academics working in similar area. We plan to establish a peer support group for researchers in dementia who use co-production. In addition, we plan to co-produce, together with unpaid carers of people living with dementia, a training course on co-production for researchers working in dementia.

RESEARCH SUPPORT / SUPERVISOR 2

Name	Dr Sarah Hotham

Institution	Centre for Health Services Studies (CHSS), University of Kent

Position	Senior Research Fellow

Description of support provided
Sarah is a Senior Research Fellow and BPS Chartered Psychologist. She is an expert in behavioural health psychology, with a specific focus on areas of nutrition (breastfeeding, obesity and malnutrition), sanitation and training of healthcare professionals. Her current and previous research has focused on three UN Sustainable Development Goals: Zero hunger; Good health and wellbeing (i.e. to decrease the risk of non-communicable diseases); and Clean water and sanitation.

Sarah has an extensive expertise in supervising projects with a focus on behavioural science approaches to design, delivery and evaluation of health behaviour change interventions. She is also interested in Implementation Science approaches to evaluation and how evidence can be translated into policy and practice.

Sarah will provide me with expertise during the Fellowship on the design of the health behaviour change intervention as well as the feasibility study. Importantly, Implementation Science is an area where I lack an experience and this is the area where I would like to gain more knowledge. Sarah will support me with this. We will meet regularly once per two months to discuss the progress of the Fellowship. She will also be involved in the outputs, and she will be member of SC.

Importantly, Sarah will also support me to think about my career beyond this Fellowship. She is based at Kent and will mentor me in advancing my own career here and beyond.

RESEARCH SUPPORT / SUPERVISOR 3

Name	Professor Naji Tabet

Institution	Brighton & Sussex Medical School

Position	Professor, Dementia and Old Age Psychiatry

Description of support provided
Naji is the Director of the Centre for Dementia Studies (CDS) and the Course Leader of the MSc Dementia Studies at Brighton and Sussex Medical School. He also leads the Dementia Theme at the NIHR Applied Research Collaboration Kent, Surrey and Sussex. He is also the Dementia Speciality Co-Lead for NIHR Clinical Research Network Kent, Surrey and Sussex. Naji has led the unit on several important phase II-IV clinical trials trying to advance the treatment of Alzheimer's disease. Importantly he is also involved in studies that investigate the role of non-pharmacological interventions, lifestyles and co-morbid physical illnesses in disease progression. He has been investigating the role of antioxidants, dietary and endogenous, and mechanistic metabolites in Alzheimer's disease and their interaction with currently available treatments. In addition to his research activities, clinically, he holds an Honorary Consultant post at Sussex Partnership NHS Foundation Trust where he delivers a diagnostic and treatment Memory Assessment Service to Trust patients with memory complaints. Naji will support me throughout the Fellowship and we will meet once per three months to discuss the progress of the Fellowship. He will also help me to think about next steps beyond the Fellowship. He will be involved in the outputs and be a member of SC. Naji is a practicing clinician and therefore I will use our conversations to better understand how people with dementia and their unpaid carers are supported in the NHS.

RESEARCH SUPPORT / SUPERVISOR 4

Name	Professor Lindsay Forbes

Institution	Centre for Health Services Studies (CHSS), University of Kent

Position	Clinical Professor of Public Health

Description of support provided
Lindsay is Clinical Professor of Public Health at the Centre for Health Services Studies (CHSS) at the University of

Kent and Theme Lead for Public Health at the NIHR Applied Research Collaboration Kent, Surrey and Sussex.

Throughout her public health career, Lindsay has worked across academia and public health policy and implementation and has extensive experience of developing and evaluating complex interventions. She has well-established links with local government, running a research network across six local authorities and leading academic input into NIHR Health Determinants Research Collaboration with Medway Council. This means that she will be able to facilitate links with local authorities, support study design, execution and analysis, and advice on dissemination strategies, particularly focusing on policymakers. In particular, Lindsay will help me focus on the next steps – how the effectiveness of the intervention can be evaluated; how it can be best implemented in the real world and how its impact on health and wellbeing can be measured. She will be part of SC.

Collaborations

I plan to undertake the award at Centre for Health Services Studies (CHSS). CHSS is a centre of research excellence which undertakes high quality research into a wide range of health and social care systems and health and care services issues at local, national and international levels. CHSS builds the evidence base for policy and practice to prevent ill-health, promote wellbeing and meet needs for care, focusing on primary and community services, public health and social care working with academic collaborators across the UK and the world. CHSS works with local government and health services across Kent, Surrey and Sussex to build research capacity and culture and address the key health and social care research needs across the region.

CHSS has a substantial number of links with other organisations including National Institute of Health Research (NIHR) Applied Research Collaboration Kent, Surrey and Sussex (ARC KSS) and the Kent Adult Social Care Partnership. ARC KSS is the national lead for social care. Primary and Community Care and Living Well with Dementia are also key themes of the ARC, with active and interlinked research programmes. There are well-established relationships between the ARC theme leads (primary care lead is Professor Harm van Marwijk in Brighton and Sussex Medical School and Professor Naji Tabet (my mentor) is theme lead for dementia). As an employee of University of Kent I have an access to senior researchers who are part of ARC and introductory emails will be sent by Professor Lindsay Forbes (my mentor) and/or Professor Naji Tabet. In addition, I will draw on a network of Professor Lindsay Forbes's local contacts in public health and primary care. Furthermore, I will join the Society of Academic Primary Care (SAPC) which would connect me to all the country's leading researchers in primary care.

Kent Adult Social Care Partnership led by Professor Ann-Marie Towers (University of Kent) supports the social care sector in Kent by conducting high quality research relating to local social care priorities. One of the key elements of the Kent Adult Social Care Partnership are Communities of Practice which bring together people who have different expertise but a shared goal. Professor Towers is my line manager and she will support me in accessing links across adult social care.

Importantly being based at Kent is also strategically important as Kent, Surrey and Sussex are in an ideal position to lead on ageing and living well with dementia research. As a region, we have a higher proportion of older people, and people living with dementia and our ageing population is growing faster than the national average. While working at University of Kent, I have formed relationships with NIHR Clinical Research Network Kent, Surrey and Sussex and independent charities such as Age UK Canterbury, Carers Support West Sussex, Alzheimer's Society (local branches) and commissioning officers based locally. I would continue to work closely with them during the award and seek their expertise and advice.

Lastly, I will be supported by the Brighton and Sussex Clinical Trials Unit (CTU) that provides support and expertise for clinical researchers in the South East in the development of funding applications, trial design, study management, quality assurance, data analysis, dissemination and trial close-down. We identified together that I will require support/mentorship in three areas: data management, study management and statistical advice.

Host Organisation Support Statement

The following statement of support must be completed by the head of your lead academic department. In order to contribute to this application, this individual must be invited to do so via the 'Participants and Signatories' section of the form.

Statement of support

12 January 2024

As Director of the Centre for Health Services Studies (CHSS), within the Division of Law, Society and Social Justice at the University of Kent, I am delighted to support **Dr Barbora Silarova's application for an NIHR Advanced Fellowship**and confirm that we will ensure she is provided with all necessary support and be able to draw on the wider services of the University in order to deliver the work in time and within budget. CHSS is a dynamic research unit delivering high quality, generalisable, applied health and care research that addresses specific health or care issues locally, regionally, nationally and internationally.

We provide a welcoming and supportive environment for Early and Mid-Career Researchers and are committed to support her employment at CHSS. We have a strong track record of supporting the development of Early and Mid-Career Researchers, many of whom have progressed their careers at CHSS and are now leading their own research projects as independent researchers. CHSS is well placed to support her proposed research through the NIHR Applied Research Collaboration Kent, Surrey and Sussex (ARC KSS) and the Kent Research Partnership, which is one of six NIHR funded social care research capacity partnerships in England.

Dr Silarova will be a full member of our staff team, currently 60 researchers, and will have opportunities to engage in all the activities run by the Centre and the University for Research Staff. She will have dedicated office space and equipment for completing her research. Two of her mentors: Professor Lindsay Forbes and Dr Sarah Hotham are based at CHSS. They will support Dr Silarova with the delivery of the Fellowship itself but also with her career beyond the Fellowship and help her to identify leadership opportunities across CHSS and at University of Kent.

Dr Silarova will benefit from being based in an active research environment, with a strong track record in health and social care, including several completed and ongoing NIHR grants relating to the needs of and outcomes of informal carers. Dr Silarova will also benefit from strong links with ARC KSS, the Personal Social Services Research Unit (PSSRU) and the NIHR School for Social Care (of which Kent is a founding member). Through the ARC KSS, she will have access to implementation, co-production and Public and Patient Involvement and Engagement support and the researcher development programmes of the ARC KSS Academy (led by Professor Kendall and Dr Julie MacInnes, also in CHSS).

Institutionally, we have an established track record of developing the careers of Early and Mid-Career Researchers to a level where they have national and international recognition as experts in the field of health and social care. The University of Kent is a signatory of the Researcher Development Concordat and supports its principles as part of our commitment to create the very best culture for our researchers. Within CHSS we also have a researcher development programme and regularly run training sessions, organise writing days and engage in a wider programme of seminars and career development activities. CHSS has a strong track record in researcher development with a clear development pathway for Early and Mid-Career Researchers.

We have been at the forefront of driving EDI activity across the organisation, establishing an Executive Lead for EDI bringing together different streams to make sure EDI is underpinned in research innovations systems and organisationally embedding an inclusive research culture. The University is building on its achievement of an Athena Swan Bronze to apply for a Silver Award, applying for AdvanceHE's Race Equality Charter and becoming a Disability Confident Leader organisation. EDI falls under the remit of the Deputy Director of Division, who ensures EDI performance is collaboratively operationalised with staff providing EDI training for selection panels, with physical and virtual environments following principles of inclusive design, and the availability of reasonable adjustments for all staff needing access to workable and sustainable arrangements.

This Fellowship will provide Dr Silarova with an opportunity to consolidate her research and develop new research programmes. She will be a member of both subject and disciplinary groups within CHSS, providing links with complementary research projects, research methods support, and external partners in the health and care community. She will have a senior researcher as a line manager (Professor Ann-Marie Towers) in addition to her mentors. Dr Silarova is a third supervisor for one of our PhD students (start of PhD September 2023; focus on unpaid carers of people living with dementia). We will identify together further opportunities for PhD supervision throughout her Fellowship.

Health and social care research are strategic priorities for the University with new investments including the Kent and Medway Medical School and an Institute for Health and Social Care launched this year. CHSS also has well established links with the health and social care community and embedded roles in NIHR through the NIHR Research Design Service South East, ARC KSS and the NIHR research capacity building Kent Research Partnership. CHSS is also working collaboratively with the NIHR Clinical Research Network and has joint research support arrangements with Kent and Medway Integrated Care Board and several NHS trusts, local authorities and third sector organisations including, for example, Kent Adult Social Care team at Kent County Council. This

provides an extensive community relevant to her proposed research. In addition, Dr Silarova will be part of the wider social science community as part of the University's Division of Law, Society and Social Justice with access to the Division's academic programmes and development opportunities.

This project has real potential to create impact in the short, medium and long-term and as director of the centre I fully support this application. Through collaborations, research capacity building and working with the wider Kent community this application will produce high impact research. The work is likely to also enable future funding for Dr Silarova to establish herself as an independent research leader. I unreservedly support this application.

9. Detailed Budget

The finance section should provide a breakdown of costs associated with undertaking the research as described in the proposal.

This section must be completed by the Applicant in conjunction with the Research Support Office or Finance Office at the host organisation.

1

Staff type	Lead Applicant

Name	Barbora Silarova

Employing organisation	University of Kent

Role on this research	Principal Investigator

Grade	U08

Starting point on scale	41

Increment Date	01/10/2024

Justification of Costs
I will be responsible for project co-ordination and management, budget management, ensuring effective communications within the team, overseeing our publication and dissemination strategy. I will be also responsible for some of the day to day activities and leading on some of the Stages/studies and dissemination outputs. I will be also responsible for co-production. It was agreed with my mentors and RSS that given my experience, proposed research and training plan I will need to work on this Fellowship for 44 months at 55% FTE.

Details of Posts and Salaries Costs

	Total (£)
Salary (£)	49205.75
Geographical Weighting (£)	
Other allowances (£)	246.03
Superannuation (£)	7134.83
Nat. Insurance (£)	5534.59
Current Annual Costs (£)	**62121.2**

2

Staff type	Support Staff

Name	Research Associate

Employing organisation	University of Kent

Role on this research	Researcher

Grade	U07

Starting point on scale	31

Increment Date	01/10/2024

Justification of Costs
Within this Fellowship application I have requested funding for support staff (research associate) to the equivalent of 50% FTE for a period of 44 months. They will be member of the Project Management Group and attend Steering committee meetings. They will be also involved in day to day activities. Depending on their preference they will be leading on some of the Stages/Studies and dissemination activities. Withour their support it would be not possible to deliver the Porposed research.

Details of Posts and Salaries Costs

	Total (£)
Salary (£)	36658
Geographical Weighting (£)	
Other allowances (£)	183.29
Superannuation (£)	5315.41
Nat. Insurance (£)	3803
Current Annual Costs (£)	**45959.7**

3

Staff type	Shared Staff

Name	Wendy Wood

Employing organisation	University of Brighton

Role on this research	CTU lead

Grade	9

Starting point on scale	47

Increment Date	01/10/2024

Justification of Costs
The CTU will provide advice, support and mentorship for database development and data management, study management and statistics.

Details of Posts and Salaries Costs

	Total (£)
Salary (£)	3844
Geographical Weighting (£)	
Other allowances (£)	18
Superannuation (£)	828
Nat. Insurance (£)	454
Current Annual Costs (£)	**5144**

4

Staff type	Shared Staff

Name	CTU Trial Manager

Employing organisation	University of Brighton

Role on this research	CTU Trial Manager

Grade	7

Starting point on scale	34

Increment Date	01/10/2024

Justification of Costs
The CTU will provide advice, support and mentorship for database development and data management, study management and statistics.

Details of Posts and Salaries Costs

	Total (£)
Salary (£)	25575
Geographical Weighting (£)	
Other allowances (£)	125
Superannuation (£)	5522
Nat. Insurance (£)	2792

	Total (£)
Current Annual Costs (£)	**34014**

5

Staff type	Shared Staff

Name	CTU Data Manager

Employing organisation	University of Brighton

Role on this research	Research Support

Grade	5

Starting point on scale	25

Increment Date	01/10/2024

Justification of Costs
The CTU will provide advice, support and mentorship for database development and data management, study management and statistics.

Details of Posts and Salaries Costs

	Total (£)
Salary (£)	25575
Geographical Weighting (£)	125
Other allowances (£)	125
Superannuation (£)	5271
Nat. Insurance (£)	2655
Current Annual Costs (£)	**33751**

6

Staff type	Shared Staff

Name	Statistician

Employing organisation	University of Brighton

Role on this research	Research

Grade	8

Starting point on scale	41

Increment Date	01/10/2024

Justification of Costs
The CTU will provide advice, support and mentorship for database development and data management, study management and statistics.

Details of Posts and Salaries Costs

	Total (£)
Salary (£)	25575
Geographical Weighting (£)	125
Other allowances (£)	125
Superannuation (£)	5271
Nat. Insurance (£)	2655
Current Annual Costs (£)	**33751**

Staff Type: (Lead Applicant)-Dr Barbora Silarova
Type of cost: HEI
% Full-time on this research: 55.0
Total months on this research: 44.0
Justification of Cost: I will be responsible for project co-ordination and management, budget management, ensuring effective communications within the team, overseeing our publication and dissemination strategy. I will be also responsible for some of the day to day activities and leading on some of the Stages/studies and dissemination outputs. I will be also responsible for co-production. It was agreed with my mentors and RSS that given my experience, proposed research and training plan I will need to work on this Fellowship for 44 months at 55% FTE.

	Year 1	**Year 2**	**Year 3**	**Year 4**	**Total Cost (£)**
Basic Salary	21650	22298	22965	11738	78651
National Insurance	2435	2525	2617	1343	8920
Superannuation	3139	3233	3330	1702	11404
Geographical weighting / Other allowances (if applicable)	108	111	115	58	392

Staff Type: Other
Other Type: Support Staff
Name: Research Associate
Type of cost: HEI
% Full-time on this research: 50.0
Total months on this research: 44.0
Justification of Cost: Within this Fellowship application I have requested funding for support staff (research associate) to the equivalent of 50% FTE for a period of 44 months. They will be member of the Project Management Group and attend Steering committee meetings. They will be also involved in day to day activities. Depending on their preference they will be leading on some of the Stages/Studies and dissemination activities. Without their support it would be not possible to deliver the Proposed research.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Basic Salary	14663	15102	15552	7949	53266
National Insurance	1521	1582	1644	846	5593
Superannuation	2126	2190	2255	1153	7724
Geographical weighting / Other allowances (if applicable)	74	75	78	40	267

Staff Type: Other
Other Type: Shared Staff
Name: Wendy Wood
Type of cost: HEI
% Full-time on this research: 2.0
Total months on this research: 44.0
Justification of Cost: The CTU will provide advice, support and mentorship for database development and data management, study management and statistics.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Basic Salary	834	878	905	458	3075
National Insurance	98	104	107	54	363
Superannuation	180	189	195	99	663
Geographical weighting / Other allowances (if applicable)	4	4	4	2	14

Staff Type: Other
Other Type: Shared Staff
Name: CTU Trial Manager
Type of cost: HEI
% Full-time on this research: 20.0
Total months on this research: 44.0
Justification of Cost: The CTU will provide advice, support and mentorship for database development and data management, study management and statistics.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Basic Salary	5639	5807	5981	3031	20458
National Insurance	610	633	657	334	2234
Superannuation	1218	1254	1291	654	4417
Geographical weighting / Other allowances (if applicable)	27	28	30	15	100

Staff Type: Other
Other Type: Shared Staff
Name: CTU Data Manager
Type of cost: HEI
% Full-time on this research: 5.0
Total months on this research: 44.0
Justification of Cost: The CTU will provide advice, support and mentorship for database development and data management, study management and statistics.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Basic Salary	1091	1116	1149	582	3938

National Insurance	108	111	115	59	393
Superannuation	256	262	270	137	925
Geographical weighting / Other allowances (if applicable)	5	5	6	2	18

Staff Type: Other
Other Type: Shared Staff
Name: Statistician
Type of cost: HEI
% Full-time on this research: 5.0
Total months on this research: 44.0
Justification of Cost: The CTU will provide advice, support and mentorship to Dr Silarova for database development and data management, study management and statistics.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Basic Salary	1733	1785	1838	932	6288
National Insurance	197	204	211	108	720
Superannuation	374	386	397	201	1358
Geographical weighting / Other allowances (if applicable)	8	8	9	5	30

Totals	Applicant	Support Staff	Shared Staff	Total Cost (£)
Total HEI	99367	66850	44994	211211
Total NHS	0	0	0	0
Total Commercial	0	0	0	0
Total Other	0	0	0	0

Travel, Subsistence and Conference Fees

Type of cost: HEI
Description: Journey Costs
Conference: False
Item Description: Travel for PI to training events
Justification of Cost: Intervention Mapping: Designing Theory-based and Evidence-based Programs: will take place in Maastricht for five days. I plan to attend three courses delivered by Gestalt Centre in London during the first year of Fellowship: Group Facilitation: Key concepts, skills and personal development (one day in person workshop); Working with Conflict and Confrontation in Groups and Teams (two days in person workshop); and Group Facilitation Training Intensive (nine days in total spread over four months). I will have to travel there from Folkestone.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	685	915	0	0	1600

Type of cost: HEI
Description: Journey Costs
Conference: False
Item Description: Travel to Focus Groups
Justification of Cost: Option will be provided to conduct focus groups either online or in-person. I will travel to focus groups if needed together with support staff.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)

	1646	1646	1646	821	5759

Type of cost: HEI
Description: Conference Fees
Conference: True
Item Description: PI conference attendance years 2 and 3
Justification of Cost: Conferences are an incredible opportunity to present one's own research, to learn about up to date evidence and make important connections. As such I plan to attend at least one conference between years 2-3 of the Fellowship. As such I will focus on conferences in three areas: dementia; health psychology/behaviour change; and ageing. Following conferences will be considered: 1. Dementia: Alzheimer Europe Conference; UK Dementia Congress; Alzheimer's Society Annual Conference; International Conference of Alzheimer's Disease International; Alzheimer's Association International Conference; Annual Liverpool Dementia & Ageing Research Conference; 2. Health psychology/behaviour change: UK Society for Behavioural Medicine Annual Scientific Meeting; the Annual Conference of the European Health Psychology Society; 3. Ageing: Annual Conference of the British Society of Gerontology

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	2000	2000	0	4000

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Travel, Subsistence and Conference Fees Total	2331	4561	3646	821	11359

Equipment
No costs have been added**Summary**

	Travel (excl Conferences)	Travel (Conferences)	Equipment
Total HEI	7359	4000	0
Total NHS	0	0	0
Total Commercial	0	0	0
Total Other	0	0	0

Consumables
No costs have been added

Patient and Public Involvement

Description: Attendance costs for monthly meetings for 8 PPI members at £55
Type of cost: Other
Please describe fully: Attendance costs for monthly meetings for 8 PPI members at £55
Justification of Cost: Attendance at monthly online meetings throughout the Fellowship. Each meeting will last 2 hours. At the moment the rate is £25 per hour. For each meeting we will contribute £5 towards their internet costs.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	5282	5282	5282	2634	18480

Description: Contribution costs for PPI members for research delivery
Type of cost: Other
Please describe fully: Contribution costs for PPI members for research delivery
Justification of Cost: In addition to attending monthly meetings, co-researchers expressed interest in conducting the research itself including conducting focus groups, reviewing all study documents and supporting dissemination activities. We agreed with co-researchers that on top of monthly meetings they may contribute up to 10 hours per month to the research delivery. The type of activities and amount of commitment will be decided with each co-researcher depending on their availability at that time as their situation and health can fluctuate. Appropriate training will be provided and was included in the budget.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	960	960	961	479	3360

Description: PPI lead output costs
Type of cost: Other
Please describe fully: PPI lead output costs
Justification of Cost: Co-researchers will contribute to all dissemination activities. However they may wish to lead on some of the outputs, e.g. working with illustrator, leaflets etc.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	857	857	858	428	3000

Description: Travel for PPI participants to conferences
Type of cost: Other
Please describe fully: Travel for PPI participants to conferences
Justification of Cost: Each co-researcher may attend two conferences throughout the whole project. At the moment we agreed together on two conferences: British Society of Gerontology and Dementia and Ageing

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	2287	2287	2286	1140	8000

Description: PPI: training for eight co-researchers
Type of cost: Other
Please describe fully: PPI: training for eight co-researchers
Justification of Cost: Co-researchers will be provided with training to supplement and enhance their existing skills so they are able to carry out research activities related to the project. All courses will be provided online and were identified together with co-researchers. Conducting online focus groups training: £220.00 per person; Digital qualitative interviews training: £220.00 per person; Writing up qualitative data training: £220.00 per person; How to search literature: £200.00 for the group; Any other training as needed: £1,000.00 for the group.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	2710	2710	2709	1351	9480

Description: PPI: Printing, stationery and postage costs
Type of cost: Other
Please describe fully: PPI: Printing, stationery and postage costs
Justification of Cost: The co-researchers will participate in monthly two hour online project management meetings. Each person will receive ahead of the meeting study packages (either printed or electronic version) including any materials that will be discussed during the meeting. Having a printed documents available was requested by five co-researchers themselves. This item includes: envelopes, large letter stamps, printed study materials and file organisers.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	358	358	358	179	1253

Description: PPI: Carer replacement costs
Type of cost: Other
Please describe fully: PPI: Carer replacement costs
Justification of Cost: Importantly to allow co-researchers to contribute we also budgeted for carer replacement costs: some carers of people living with dementia may require having someone in place instead of them to look after a person living with dementia while working on the project. The co-researchers I worked with did not require this in last twelve months however their situation may change in the upcoming years. Four co-researchers are former carers so they will not require these costs.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	2081	2081	2081	1037	7280

Description: PPI: attendance at Dementia and Ageing conference
Type of cost: Other
Please describe fully: PPI: attendance at Dementia and Ageing conference
Justification of Cost: Each co-researcher will be able to attend two conferences throughout the Fellowship. Two

conferences were agreed together with co-researchers: British Society of Gerontology and Dementia and Ageing attendance.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	126	126	126	62	440

Description: PPI: attendance at BSG conference
Type of cost: Other
Please describe fully: PPI: attendance at BSG conference
Justification of Cost: Each co-researcher will be able to attend two conferences throughout the Fellowship. Two conferences were agreed together with co-researchers: British Society of Gerontology and Dementia and Ageing attendance.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	229	229	228	114	800

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Patient and Public Involvement Total	14890	14890	14889	7424	52093

Other Direct Costs

Description: EDI: transcription costs for one to one interviews
Type of cost: HEI
Please describe fully: EDI: transcription costs for one to one interviews
Justification of Cost: If people do not wish to participate in a focus group, e.g. due to a language barrier, an option will be provided to have an individual interview with an interpreter present. These interviews will be then professionally transcribed.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	110	110	110	55	385

Description: EDI: Carer replacement costs for project participants
Type of cost: Other
Please describe fully: EDI: Carer replacement costs for project participants
Justification of Cost: We will contribute to carers' replacement costs (£25 per hour) so participants can take part in focus groups while a professional is supporting a person with dementia.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	393	393	393	196	1375

Description: Ethical approval costs 3 x £240
Type of cost: HEI
Please describe fully: Ethical approval costs 3 x £240
Justification of Cost: 3 x £240 (inc. VAT) for research approval applications that are put forward for consideration to the ADASS Research and Branding Subcommittee. We will apply for research approvals for Stage 1, Stage 2 and Stage 3.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	192	192	192	0	576

Description: Encrypted dictaphone
Type of cost: HEI
Please describe fully: Encrypted dictaphone
Justification of Cost: Focus groups will be audio-recorded using encrypted digital voice recorders. I included budget for one encrypted digital voice recorder.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	336	0	0	0	336

Description: Endnote licence
Type of cost: HEI
Please describe fully: Endnote licence
Justification of Cost: To support conduct of systematic review.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	180	0	0	0	180

Description: KMTV dissemination costs
Type of cost: HEI
Please describe fully: KMTV dissemination costs
Justification of Cost: KMTV (TV made for Kent) will produce a 12-minute UK based documentary with The primary aim of this output is to disseminate the research findings and to engage policy makers and media stakeholders. It will be transmitted on the Local Television Network Channel 7 through KMTV, together with the coordination of associated press and publicity campaigns undertaken with partner organisations.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	2744	2744	2744	1368	9600

Description: Laptop for support stuff
Type of cost: HEI
Please describe fully: Laptop for support stuff
Justification of Cost: Within this Fellowship application I have requested funding for support staff. Funding for their laptop was included.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	800	0	0	0	800

Description: Pay as you go mobile
Type of cost: HEI
Please describe fully: Pay as you go mobile
Justification of Cost: Within this Fellowship application I have requested funding for support staff. They will need a mobile phone to be able to be get in touch with participants directly.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	20	0	0	0	20

Description: Data collection: Study packs printing and stationery
Type of cost: HEI
Please describe fully: Data collection: Study packs printing and stationery
Justification of Cost: Some participants may prefer to request information in paper form rather than online. They will also have an option to fill in the survey in Stage 1a either in paper form or online. This item includes: prepaid envelopes, stamps for large letters and printed documents.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	1043	1043	1042	520	3648

Description: EDI: documents translation up to 3 languages
Type of cost: HEI
Please describe fully: EDI: documents translation up to 3 languages
Justification of Cost: As part of recruitment/data collection documents will be translated into three additional languages.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	686	686	686	342	2400

Description: Thank you vouchers for participants
Type of cost: HEI
Please describe fully: Thank you vouchers for participants
Justification of Cost: We will provide Thank you vouchers to participants taking part in focus groups (Stage 2b, 2e and Stage 3) . £25 voucher per participant. Participants will receive £25 voucher irrespective of their professional background.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	572	572	571	285	2000

Description: Transcription costs for Stage 2
Type of cost: HEI
Please describe fully: Transcription costs for Stage 2
Justification of Cost: Focus groups will be audio-recorded using encrypted digital voice recorders and transcribed.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	234	234	233	116	817

Description: Transcription costs for Stage 3
Type of cost: HEI
Please describe fully: Transcription costs for Stage 3
Justification of Cost: Focus groups will be audio-recorded using encrypted digital voice recorders and transcribed.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	117	117	116	58	408

Description: EDI: Interpreter
Type of cost: HEI
Please describe fully: EDI: Interpreter
Justification of Cost: If people do not wish to participate in a focus group, e.g. due to a language barrier, an option will be provided to have an individual interview with an interpreter present.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	274	274	274	137	959

Description: Zoom licence
Type of cost: HEI
Please describe fully: Zoom licence
Justification of Cost: Co-researchers struggled with MS Teams when we worked together on this application and it was suggested that Zoom would be better alternative for our meetings. As we plan to also conduct focus groups online with carers of people living with dementia we agreed that Zoom will provide participants better experience.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	104	104	104	104	416

Description: NHS research costs from SOECAT
Type of cost: NHS
Please describe fully: NHS research costs from SOECAT
Justification of Cost: These are the research costs approved by CRN

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	193	192	192	83	660

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Other Direct Costs Total	7998	6661	6657	3264	24580

Training And Development

Type of cost: HEI
Description: Short Courses
Item Description: Training and development: support staff
Justification of Cost: Within this Fellowship application I have requested funding for support staff. We will also identify together any training and development needs. Therefore, £3000 was requested to support them. The examples of training can include: conferences; specific skills and knowledge needed for the proposed project; specific skills and knowledge needed for the next stage in their career etc.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	857	857	858	428	3000

Type of cost: HEI
Description: Short Courses
Item Description: Training focused on Behaviour Change Wheel
Justification of Cost: The proposed research is informed by Behaviour Change Wheel. During the Fellowship I would like to gain more in depth-knowledge of Behaviour Change Wheel and therefore I propose to attend two Advanced Modules in Behaviour Change: Principles and Practice of Behavioural Systems Mapping (£400) and the Behaviour Change Intervention Ontology (BCIO): How to use the BCIO for different research and applied purposes (£400). I plan to attend these courses during the first year of the Fellowship. These courses are delivered by the developers of the model based at Centre for Behaviour Change, UCL. In addition to gaining a specific knowledge, attending these courses will also allow me to connect to other researchers interested in this specific approach.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	800	0	0	0	800

Type of cost: HEI
Description: Short Courses
Item Description: Training focused on behaviour change interventions: Intervention mapping
Justification of Cost: Behaviour Change Wheel represents one of the approaches to behaviour change. There are other approaches to developing interventions e.g. Intervention Mapping which provides a framework for developing theory-based and evidence-based interventions applied to health promotion and disease prevention. To be able, in the future, to support other researchers aiming to improve the wellbeing and health of unpaid carers of people with dementia, I also propose to attend a 5-day summer course on Intervention Mapping: Designing Theory-based and Evidence-based Programs at Maastricht University in the Netherlands. Being trained in two key approaches to behaviour change intervention development will enable me to work beyond this Fellowship on a range of projects and connect with people who apply these approaches in their work. I plan to attend this course in the second year of the Fellowship.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	2500	0	0	2500

Type of cost: HEI
Description: Short Courses
Item Description: Training in Co-production
Justification of Cost: As a researcher I am committed to work in a co-produced way together with unpaid carers of people living with dementia, other academics, health and social care professionals. While co-producing this application over the period of twelve months together with unpaid carers I have identified two key areas where I need additional training: 1. working with conflict and confrontation in teams; and 2. facilitation skills. As such I plan to attend three courses delivered by Gestalt Centre in London during the first year of Fellowship: Group Facilitation: Key concepts, skills and personal development (one day in person workshop); Working with Conflict and Confrontation in Groups and Teams (two days in person workshop); and Group Facilitation Training Intensive (nine days in total spread over four months).

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	2805	0	0	0	2805

Type of cost: HEI
Description: Short Courses
Item Description: Impact and dissemination training
Justification of Cost: I aim to become a researcher who has an influence beyond academia. Specifically, I would like to make the needs of unpaid carers visible to present and future health and social care professionals as well as policy makers. I plan to attend the following courses: a) Impact Training for Researchers (free online course provided by Fast Track Impact over 5 weeks; first year of the Fellowship) and Compelling Communication Skills (£2050; 6 week course delivered by University of Cambridge; second year of the Fellowship).

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	2050	0	0	2050

Type of cost: NHS
Description: Short Courses
Item Description: Software skills
Justification of Cost: I would like to become an expert on two software packages I regularly use in my research and will be also central to delivery of this Fellowship: NVivo and Qualtrics. As such I suggest attending two courses: NVivo Expert Certification (£1000): this will enable me to be an in-house NVivo expert and advisor for other researchers across University of Kent. I will also do Qualtrics online courses (free), again allowing me to raise my profile in the research community.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	1000	0	0	0	1000

Type of cost: HEI
Description: Short Courses
Item Description: Specific skills and knowledge needed for the next stage of the project
Justification of Cost: Economic Evaluation Online: An introductory course; third year of the Fellowship.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	0	0	180	0	180

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Training And Development Total	5462	5407	1038	428	12335
	Year 1	**Year 2**	**Year 3**	**Year 4**	**Total Cost (£)**
Total Direct Costs	88779	91409	87951	43439	311578

Indirect costs

HEI Indirect costs

Type of cost: Estates Charges
Name of Higher Education Institute: University of Kent
Justification of Cost: University of Kent indirect costs calculated using TRAC methodology.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	9727	9727	9727	4863	34044

Type of cost: Other Indirect Costs
Name of Higher Education Institute: University of Kent
Justification of Cost: University of Kent indirect costs calculated using TRAC methodology.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	57893	57893	57893	28946	202625

Type of cost: Estates Charges
Name of Higher Education Institute: Brighton and Sussex CTU
Justification of Cost: Brighton and Sussex CTU indirect costs calculated using TRAC methodology.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	673	673	673	334	2353

Type of cost: Other Indirect Costs
Name of Higher Education Institute: Brighton and Sussex CTU
Justification of Cost: Brighton and Sussex CTU indirect costs calculated using TRAC methodology.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
	1886	1886	1886	936	6594

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
HEI Indirect costs Total	70179	70179	70179	35079	245616

Commercial Indirect Costs
No costs have been added

Other Partner Organisation Indirect costs
No costs have been added

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Total Indirect Costs	70179	70179	70179	35079	245616

Research Costs Requested From Funder

	Direct costs	Indirect costs/Overheads	Amount requested
Total Higher Education Institution Costs (80%)	241115	245616	486731
Total Higher Education Institution Costs (100%)s	15335	0	15335
Total Commercial Costs	0	0	0
Total Other Parther Organisation Costs	53468	0	53468
Total NHS Research Costs	1660	0	1660
Total Research Costs Requested From Funder	311578	245616	557194

Summary

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Salaries	**58098**	**59890**	**61721**	**31502**	**211211**
Travel-subsistence-and-conference-fees	**2331**	**4561**	**3646**	**821**	**11359**
Equipment	**0**	**0**	**0**	**0**	**0**
Consumables	**0**	**0**	**0**	**0**	**0**
Patient-and-Public-Involvement	**14890**	**14890**	**14889**	**7424**	**52093**
Other-Direct-Costs	**7998**	**6661**	**6657**	**3264**	**24580**
Training And Development	**5462**	**5407**	**1038**	**428**	**12335**
HEI-Indirect-costs	**70179**	**70179**	**70179**	**35079**	**245616**
Commercial-Indirect-Costs	**0**	**0**	**0**	**0**	**0**
Other-Partner-Organisation-Indirect-costs	**0**	**0**	**0**	**0**	**0**
Total	**158958**	**161588**	**158130**	**78518**	**557194**

NHS Support Costs

Cost Item:	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
General activities	495	495	495	495	1980.0

Justification:
General activities: Stages 1a, 2a, 2b, 2e and Stage 3 will be supported by CRN. The staff across CRN will help me to identify potentially eligible participants (database search and eligibility check, both NHS Support costs) and they will also mail out any relevant invitation documents (this is research cost). All other activities will be conducted by the research team. We agreed with our CRN lead that to meet our recruitment target across each Stage/study we will have to engage with 30 NHS sites and the target for recruitment across all stages for NHS sites is 260. Please note additional recruitment pathways will be used too: e.g. adult social care including third sector organisations etc.

	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
NHS Support Costs Total	495	495	495	495	1980

Have you discussed and agreed these costs with the Lead Network?	Yes

NHS Treatment Costs
No costs have been added

Have you discussed and agreed the NHS costs with the Lead Trust?	N/A

Is the patient/service user care being provided different from the usual standard treatment for the condition?
N/A

Usual Treatment Costs
No costs have been added**Excess Treatment costs/(savings)**

	Cost per patient(£)	Year 1	Year 2	Year 3	Year 4	Total Cost (£)
Total NHS Excess Treatment costs/(savings)	0	0	0	0	0	0

NHS Support and Excess Treatment Costs

	Total (£)
NHS Support Costs requested from networks	1980
NHS Excess Treatment costs/savings	0
TOTAL NHS SUPPORT AND TREATMENT COSTS	1980

Total Funding Required (including non-research costs)

	Total (£)
Total Research Costs Requested from Funder	557194
Total NHS (Support and Excess Treatment) Costs	1980
Total Funding Required	559174

10. Uploads

(References - References.docx) is included as an appendix within this file.

(Figures / Tables - Figure 1. Research plan.docx) is included as an appendix within this file.

(Research Timetable - Research timetable.docx) is included as an appendix within this file.

(CTU Letter of Support - BSCTU letter of support Barbora Silarova.pdf) is included as an appendix within this file.

(SoECAT Template - Funder Export_339107_SoECAT_Healthy you – keeping yourself healthy while supporting someon.xlsx) is included as an appendix within this file.

11. Participants and Signatories

Devolved Administrations

no Devolved Administrations Participant has been added

Post-Doctoral Primary Supervisor

Research Support

Research Support Participant

Title	Dr
Forename(s)	Rosalie
Surname	Ashworth
Position	Partners in Research Lead (Patient and Public Involvement)

Research Support Participant

Title	Professor
Forename(s)	Lindsay
Surname	Forbes
Position	Clinical Professor

Research Support Participant

Title	Dr
Forename(s)	Sarah
Surname	Hotham
Position	Senior Research Fellow

Research Support Participant

Title	Professor
Forename(s)	Naji
Surname	Tabet
Position	Professor and Director

Host Organisation - Heads of Department

Head of Department or Senior Manager

Title	Professor
Forename(s)	Lisa
Surname	Dikomitis
Position	Professor of Medical Anthropology and Social Sciences

Administrative Authority or Finance Officer

Administrative Authority or FO Participant

Title	Mrs
Forename(s)	Amanda
Surname	Burns
Position	

12. Acknowledgement, Review and Submit

Conflict checks:
Please declare any conflicts or potential conflicts of interest that you may have in undertaking this research, including any relevant, non-personal & commercial interest that could be perceived as a conflict of interest.

Nothing to declare.

Agreement to Terms and Conditions:

I confirm that I have secured all necessary licences and ethical approvals in relation to the research and will abide by the terms of those licences and approvals in carrying out the research.

I confirm that I have read the NIHR Carbon Reduction Guidelines and, where possible, taken steps to reduce the carbon emissions generated by this research.

I confirm that all material within this application, with the exception of sections that other participants are required to complete, is my own work and I have taken steps to ensure that no parts of the application have been inappropriately plagiarised.

I undertake to conduct the research according to such guidelines for good research practice as the Host Organisation may from time to time laydown according to the NIHR Research Governance Framework.

If you are applying for an NIHR award which comes with partnership funding from another organisation (e.g. a charity or industry partner) your information (including your NIHR Application Form and subsequent progress/annual reports) may be shared with the partner organisation as part of the review process and ongoing monitoring of the award.

In completing this, I declare that, to the best of my knowledge, the information provided in this application, is true, accurate and complete. I have read the Guidance Notes and agree to accept the process by which an application is assessed and agree to abide by the conditions under which an award may be granted.

☒Confirmed

Appendices

1. References.docx
2. Figure 1. Research plan.docx
3. Research timetable.docx
4. BSCTU letter of support Barbora Silarova.pdf
5. Funder Export_339107_SoECAT_Healthy you – keeping yourself healthy while supporting someon.xlsx

Appendix #1: References.docx

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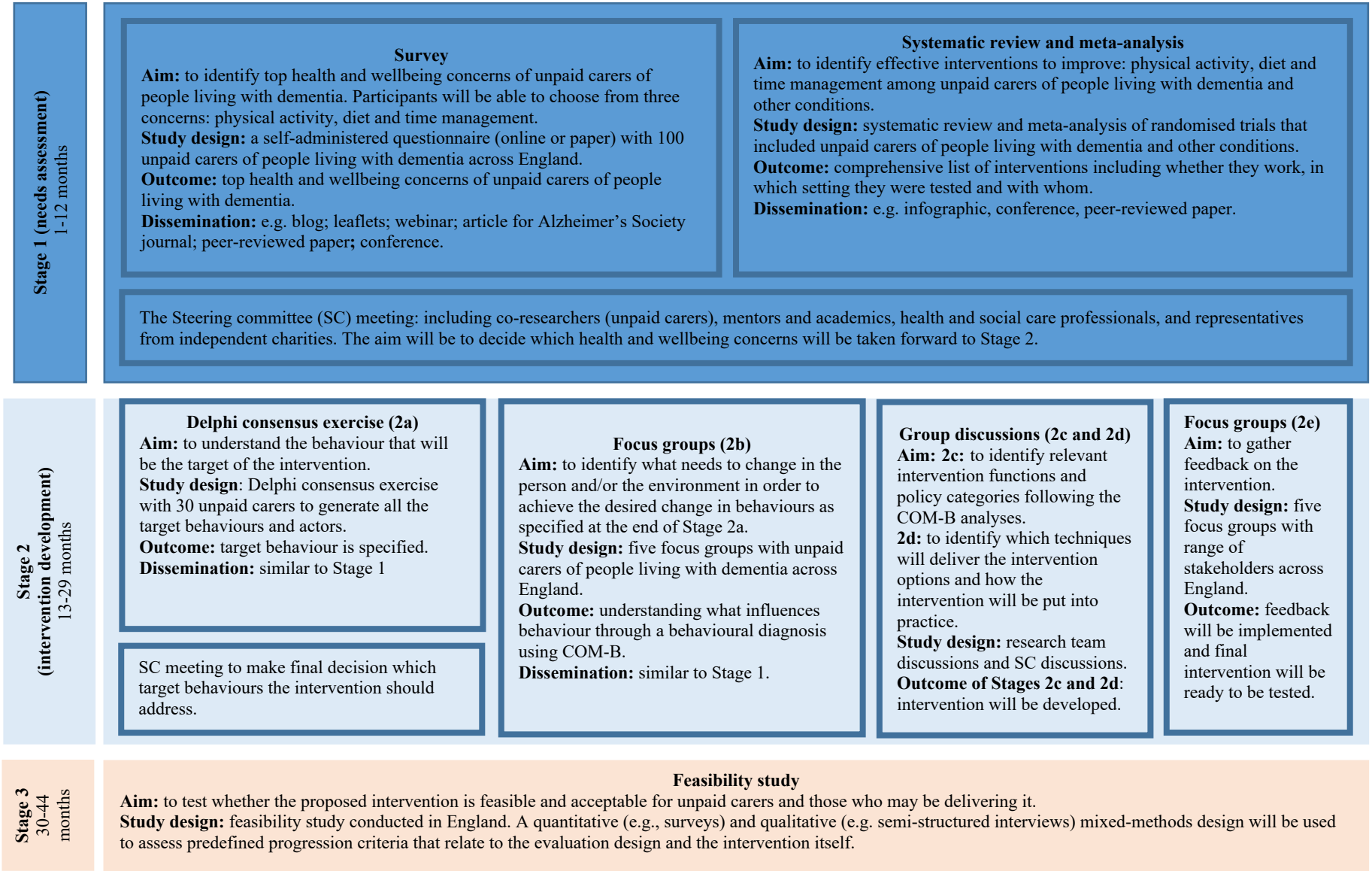
Appendix #2: Figure 1. Research plan.docx

Figure 1. Research plan



Appendix #3: Research timetable.docx

Research timetable														
Start of the project: 1st January 2025; End of the project: 31st August 2028; In total: 44 months														
	1 -3	**4-6**	**7-9**	**10-12**	**13-15**	**16-18**	**19-21**	**22-24**	**25-27**	**28-30**	**31-33**	**34-36**	**37-39**	**40-44**
Project management														
Set up governance groups														
Steering Committee														
Stage 1: Needs assessment; 1-12 months														
Ethical approval for the survey														
Development of protocol for the review														
Data extraction/analysis for the review														
Recruitment of participants for the survey														
Data collection and analysis of results from the survey														
Dissemination of results														
Stage 2: Intervention development; 13-29 moths														
Ethical approval for Stage 2														
Stage 2a: Understanding the behaviour; Delphi exercise														
Recruitment and conduct of Delphi exercise														
Data analysis from Delphi exercise														
Dissemination of results														
Stage 2b: Specify what needs to change														
Focus groups: recruitment and conduct														
Focus groups: data analysis														
Dissemination of results														
Stages 2c) Identifying intervention options; research group discussions														
Stage 2d) Identifying content and implementation options; research group discussions														
Stage 2e) Gathering feedback on intervention														
Focus groups: recruitment and conduct														
Focus groups: data analysis														
Dissemination of results														
Stage 3: Feasibility study; 30-44 months														

Appendix #4: BSCTU letter of support Barbora Silarova.pdf

Brighton and Sussex
Clinical Trials Unit
brighton and sussex
medical school

Brighton and Sussex CTU
Room 110
Watson Building
University of Brighton
Brighton
BN1 9PH

11th January 2024

Re: Dr Barbora Silarova NIHR Advanced Fellowship

To whom it may concern

The Brighton and Sussex Clinical Trials Unit (UKCRC registration ID 66) are delighted to be able to support Dr Silarova in carrying out the research study (Healthy you – keeping yourself healthy while supporting someone living with dementia: development and feasibility testing of a complex intervention) as part of her NIHR Advanced Fellowship.

The CTU will provide advice, support and mentorship to Dr Silarova for database development and data management, study management and statistics.

We look forward to working with her on this project.

Yours sincerely

Dr Wendy Wood
Director of BSCTU

Appendix #5: Funder Export_339107_SoECAT_Healthy you – keeping yourself healthy while supporting someon.xlsx